Two Pharma Giants Who'd Never Met | Mankind & Dr. Reddy's | WTF is Pharma?

India makes medicines that heals billions of people across the world. Most of them have no idea who's behind it. After much demand, we brought together two of India's pharma giants, Dr. Reddy's and Mankind to help you, the young entrepreneurs of this country, grasp the industry's nuances.

Their products are household names, probably consumed by you at some time, and of course consumed by millions. So we decided to put a face to these stalwarts. So how do you talk yourself into it every day? Supposed to prove it to my family, to my brother, that I'm worth something.

So the talk was only one that, how can I make it to a level that I can start respecting myself? And I'm sure you've been through multiple phases of going through tough times. The biggest challenge I ever had was in our operations. This happened in the mid last decade, in the middle of the last decade, around, we had safety incidents where people died and that really shook me up and really put me in a deep state of depression.

So that was a very difficult time for me. Reddy Pharmaceuticals is headed by G. V. Prasad and brings in roughly 3 billion in revenue. Mankind is led by Rajiv Juneja and his brother, with an annual turnover of about 40,000 crores.

With numbers like these, many would coast, but not Rajiv or G. V. Their hunger keeps pushing the boundaries. Here's the thing nobody tells you, India's called the pharmacy of the world, but that's by volume. By revenue, we're barely a rounding arrow.

We make the drugs, but someone else makes the money. So who and why? And more importantly, where does a 25-year-old find an opening in all of this? That's exactly what we asked. What is the business opportunity here? Can you guys think of anything else for the 25-year-old? There are 1 in 28 unicorns in India today, there is one in pharma drug innovation.

This is very casual, I was telling him earlier, Prasad, that our audience is largely young people who want to start a business in your industry, so we won't speak so much about your company in particular, there's nothing controversial, there's no headline, it's very much everything a 25-year-old needs to start a business in the pharma industry. So both of you are professors today, and you are teaching a class of young 25-year-olds. I'm part of that class, so I will keep asking you questions.

Before we start, maybe you guys can tell me a bit about yourselves, I know you guys are very successful, popular people, but a little bit about your life and how you got to be where you are. Would you like to go first? I can go first, I don't have any issues. My name is Rajiv Juneja, and I have one wife, and I have five kids, three daughters, two sons, I mean daughters means, I mean

they are daughter-in-laws, and one grand-daughter.

So I come from a place called Meerut, which is 60 kilometers away from Delhi, I'm new in Delhi, 2004-05 we came to Delhi, and before that my education, my brought up, all was in Meerut, and I'm a college dropout, because circumstances were like this, and number one, I was not interested in studies, and so that's why I got an excuse that there's a compulsion in the family, and the money is not there. So I used to go to my brother's medical store, there I learned how to run a pharmacy, like people go to Nani's place in summer vacation, I used to go to that medical store for two months. So eight, nine months I worked over there, that was from 10th class to POC part one.

Then one day I said, I want to be a medical rep, I became a medical rep in my brother's company, worked 10 years over there, and there I did a lot of things, and started a company called Mankind, in 1995. 1995? 95, yeah. It's been that long? Yeah, 30 years.

Wow. It's been a while. Just one second.

There's no chain name at Pablo, don't, leave him open, I'm telling him don't keep the chain. Okay let's continue. Today, whatever mankind is, is because of whatever I'll say, destiny dictates.

How did you go from pharmaceutical school to medical school? With your experience of working in a pharmaceutical store and starting lines, what worked? See what basically happens in life, what you hate, becomes a big lesson for you. I used to hate going to the pharmacy, cleaning, sweeping, buying goods from wholesale market. I mean, at the age when my friends were enjoying.

So that was the lesson, how market really work, how sales reps, how companies, how doctors operate, how this market operates. I never knew that. And then I became a medical rep in every local kind of a company.

It was local means in the sense that like black and white company, not even colors were there in boxes and visual aids or literatures. So when you work in that kind of a company, if you will not become a brand yourself, company will not run, where failure rate is 80-90%. Every time just, you become so thick skinned, at the same time sensitive.

I mean, it was somewhat oxymoron. Same time you are thick skinned, same time you are very, very sensitive. When you go through that kind of a phase and these two enable that, one time that company start which was started with Rs.69,000 in 1984, became Meerut's number one company.

We had beaten Black Sores, Cibla, any company, everybody, every company, sheer hard work, sheer hard work. That's it. I mean, har nahi mani, karna hi, tarna hai, choice hi kuch nahi hai.

When you have this kind of grit, something good really happens. We were almost Western UP's top 10 companies. And then some family separation was there.

So me and my brother said, we'll start a new company and we got Rs.50 lakhs. And in those days, Bestow Care was 10 crore revenue company. That's just a very peaceful partition and we wanted to get away.

Every learning was not in mankind. Every learning happened during that time of huge struggle. Mankind today is fourth largest pharma company, the revenue of more than Rs.40,000 crore.

We are the company which operates in veterinary side as well. We operate in pet side as well. We are an OTC company as well.

What's the biggest part? The biggest part is pharma. Biggest part, say, 85% is pharma. Is there one particular product you get? One product which is quite a big one is TellMeKind.

It's for hypertension. Rs.1000 crore. Second brand would be Manforce.

That's quite a known name. Condoms. Around together Rs.600 crore.

That is what Mankind is. Mankind is basically, people say, it's an OTC kind of a company because of Manforce. But if you just look and go deeper down, it's a pharma company.

Intentionally, we started OTC because we wanted to be known. When you start very small, we started from Meerut. We never ever started All India and Mango.

First, Western UP only. First made a try. And slowly and gradually, Eastern UP, then Haryana, then Punjab.

Basically, evolved. Wherever we went, it's like we were a small company rising very fast. So, after all this, what you basically see is respect.

Name. And how can you get the name? Then you come on television. How can you come on television? As a pharma company, you cannot come.

So, it launched. Your OTC company would be there. In the end, your logo would be displayed.

And we used to keep logo for a longer period. Otherwise, just generally, it's half a second. We used to keep it for more than a second.

That was the strategy. Magazines, newspaper, every place. So, actually, we basically wanted to propagate Mankind all over India.

Before Mankind is to be launched any place. South side, we were not there. We came to South 19 years back.

We are 30 years old. But it was all gradual. We were not in a hurry.

Our India was UP. Our India became Haryana and Punjab. And then UP.

Our India. We never thought that India is one. All focused.

It was always very focused approach. Deepest coverage. And he is a scientist.

He is a different guy. He comes from that top side of the pyramid of Pharma. R&D and other things.

I always used to think of you guys like commandos. Come in and push forward all the competition and go ahead. We basically call ourselves FMCEP.

FMCG of Pharma. I mean, we had nothing. Same, branded, generic.

No, but you are very unconventional. How you got into the market. How you pushed out.

Never followed anything of the industry. Never. Absolutely contrarial.

100%. And even right now, if everybody is doing something, I will not do. If something is, I will do it.

What are you most bullish about? G part of FMCEP or BFM? I am most bullish now. I mean, as you basically evolve. We are going to come in this category.

In which category? In this category. These guys get very young. R&D guys.

That's our next goal. And do you guys interact? First time. Never.

Wow. We go in and sit. It's mosquito time.

15 minutes, there will be mosquitoes. It's Sunday here. But I mean, if you just ask me, mosquitoes in Delhi versus this place.

More mosquitoes. I can't see. No, they come.

Strangely, at one particular time. At 5.45. They suddenly come. And then at 6.30, they are not there.

Which is very strange. I don't know where they come from. But come.

What's up? Change? Huh? Change? Hmm? Hmm? I have seen a lot like this. So, for starters, I wanted to understand. What did your first day in Parwan look like? Nervous.

Bad. Very bad. I ran away.

I... I lied to my brother that I walked. Because I had no confidence. In meeting, going in a medical college.

That was the first thing I said. What did your 1980s look like? Kind of wild. I was in college till 82, 83.

82, 83, I was doing my masters. Decided not to study. Yeah? Experienced everything that life had to offer.

So, went to concerts, went to games. That's how my 83 was. I came back.

Joined my family business, which was in construction. But also I started an entrepreneurial venture in pharmaceuticals. Ran that for some time.

It didn't do too well. Then I exited. And what is something in pharma that you have seen from 1980s to now that has changed so significantly in India? I think, you know, when we started our business, you just had to make the product to make money.

Now we have to be very good, excellent, highly competitive. The business has become very crowded. There are not many people here.

And the whole thing has scaled. What was that day when you thought you have arrived? Maybe third year. Third year.

And what do you think changed that day? I found few mentors, actually, to be honest. I can call them mentors now. In those days, they were my gurus or whatever you call it.

I made a choice that I should be in the company of smart people. And did something, you know, significantly shift when you say, okay, I have made it now, and I don't need to be worried? What was that moment? Like, did you buy something? Did you have access to something that you didn't have before? What was that? Till now, that day has not come. Have you always loved science? I always liked science.

Interesting. I really loved it because, you know, it's like magic. You take simple molecules, combine them to make complex molecules, and suddenly this molecule cures a disease.

And what made you come back? I was always... I never thought I'd settle in the U.S. Really? Yeah. I mean, that's so different for so many people who grew up in that era. I think business families generally expect their children to come back, so I was kind of... And what would your advice be for someone who's growing up in a business family? At that time, actually, I wanted to do a PhD, but I got admission, but my father's business, he was going through some trouble and asked me to come back and join him.

I'd ask people now to do what they love to do, what their dream is. Fate has its own way. Mr. G.

V. Prasad came back to India to run the family construction business.

He then went on to start his first pharmaceutical venture with his father. Their factory was right next to one belonging to Dr. Anji Reddy, who was already a successful entrepreneur. Around this time, he got married to Anuradha Reddy, who was Dr. Reddy's daughter.

In 1988, the new pharma venture was not scaling, and his father-in-law, growing fast and needing capacity for Dr. Reddy's laboratories, bought the plant and merged it with his. G. V. Prasad moved on, went back to construction, and then one day, the phone rang. Dr. Reddy was running two companies at the time, Cheminor Drugs and Dr. Reddy's Laboratories.

The co-founder of Cheminor Drugs had parted ways due to differences. Dr. Reddy needed someone he could trust. G. V. Prasad said yes without thinking.

That was 1990. He became the CEO of Cheminor Drugs. The group was doing under 100 crores.

He ran one company, built it, scaled it, and in 2001, Cheminor merged with Dr. Reddy's and was listed on InBiase. Before Dr. Anji Reddy passed, he wrote a book. He called it The Unfinished Agenda, a dream of discovering new molecules, new cures, a truly Indian pharmaceutical legacy.

He didn't live to see it completed. The effort still continues through Origine Oncology. So what would you say is the one differentiating advice that you would give to someone who's building in this space? In pharma, I'd say be careful which part of the business you're entering.

Enter a space where there is room to play, grow, and mistakes can be tolerated. Focus on where your strength lies, where your competitive advantage will be, what problem that you're solving in a unique way, and get into that. Think through all this before you jump in.

These are a little more popular, actually. Oh. Suza and Jamli Roy's.

My favourite is that one. The one you showed me. Suza is quite rare because normally he paints portraits.

He doesn't paint full-size. It's very interesting because you look at somebody who's like a handicapped man being carried, and you look at the man carrying the handicapped person who's also handicapped. It's quite interesting.

It's kind of saying to society that people who look strong are also handicapped. He's a quite popular one. Suza is known.

These are all popular. Suza, Jamli Roy. Vaikuntham.

Vaikuntham is my city. That is the last supper. But all the characters have been changed to artists.

Oh, really? Yeah. You see M.F. Hussain, you see Dali. Oh, really? Yeah.

This is very interesting. Dali is in the centre. There's Rembrandt, there's Van Gogh, there's Andy Warhol.

It's a paint or what? No. It's a graphite pencil. Who's the guy? This is Jamli Roy, right? Yeah, this is Jamli.

Very good. Who are these people? These are Italian painters from 300 years ago. 1700s.

He's a master but his name is not attributed. It's like unverified. That's one Kiran gave me actually.

This one? Yeah. Oh, this one? The one above it. So Kiran's husband.

Looks like an Irish one. Correct. So John.

Yeah, John. This is from somewhere near John's house. If you just look at the paintings and if nobody tells you he's an artist, what is the meaning? A black canvas is great.

I have a lot of stuff to ask you guys. Who did the research from your team? Rushan. He told me.

Yeah. Rushan did the research on this one. Okay, so you said you want to start with establishing the Indian pharma industry, then we get into the different molecules and… Different modalities.

Yeah. Okay. So, let me start with segmenting the industry first.

And then we'll talk about the evolution of the segment and I think Rajiv… We'll focus on Rajiv more for business and Professor Prasad more on science. Even I'm learning from him. Certain things, I mean I keep talking to my R&D guy and he explains me things.

Then it really goes away because my mind is always there somewhere else. Right. Actually, I'm not a scientist also.

I'm a business guy. See, I mean, somebody who has spent so many years over there in majorly science side, you become knowledgeable. Yeah.

Yeah. For me, you're a scientist. Yeah.

So, let me start with the segmentation. Okay. Broadly, the industry has four or five verticals, if you call them in IT language.

API, Active Ingredient. You want to start by biggest vertical to smallest? From an Indian context, the biggest one is pharmaceuticals. Okay.

Generic pharmaceuticals. Generic Pharma is the biggest. Yeah, they're branded Generics

actually.

It's the biggest vertical. It's actually copies of products which were discovered, innovated by major pharmaceuticals around the world. We make them equivalent to them.

And there's a whole study on how we manage equivalence. I don't know if you want to know that. So, you explain how a drug is discovered.

Yeah. Then you explain patent. Yeah.

So, once the patent runs out, then the generic starts? Even before it runs out, you can do the R&D work. Keep it ready for the day the patent expires and then sell it. How does an American, I'm just saying American, European, whatever, company decide which pharmaceutical company in India, China, whichever country do they give the contract to? They don't get anything.

They don't have a say on this. Anybody who wants to do it after the patent expires is free to do it. So, it's like this.

The innovator discovers drugs. He has a protected monopoly for a few years. Yeah, it depends.

Once the patent runs out, anybody in the world can develop an equivalent product. They have to get it registered with the health authorities that it is really an equivalent product. And then they can develop the product, get it registered and sell it.

How do you register? The health authority will? Yeah, I'll tell you how that is done. So, you have a product discovered by Pfizer. Or you say semaglutide discovered by Novolaz.

It's an injection. Injections are simpler in some ways because you just have to establish analytically that they are the same solution that you are putting in the body. Sometimes they are not.

Then you have to do studies on humans. Tablets, you have to establish that the release of the drug in your body is equivalent to the way the innovator's product is released in the body. When you take a pill, it goes into your digestive tract.

It passes the thing and it goes into your bloodstream. And then ultimately the liver kind of cleanses it out of your system. But in this interim, the blood takes it to where it's supposed to go.

And the ingredient acts on the cells that it needs to act and cures your disease. It's like a lock and key. The protein is there and your drug goes there and corrects whatever needs to be corrected.

So, there is a concentration of the product in your blood. When you take a tablet for the next 24 hours, 48 hours, whatever, it goes up. The concentration goes up.

Then it's cleansed out of your system. So, this is called the concentration curve. You have to

match that.

Is that a big science, when it is released? Not really. I mean, it's pharmacokinetics. For a person in the heart, it's not so complicated.

You develop a formulation, which is… And that binding material, the quality is significantly different from a… No, no, no. The binding doesn't happen because of anything extraneous. The product itself is designed to bind.

So, the molecule, it's like you have a lock. It's a key, which goes in and turns. Right.

Does the… Then what… Like, say, for example, there is one drug, which should dissolve after, I don't know, like 30 seconds. One more after three minutes. So, what changes that? The way you formulate the product.

The quality of the… The characteristics of the substance and the way you formulate it. How you make a tablet. Right.

How do you make it? With what do you make it? Active ingredient is one part of it. Then you have inactive ingredients. You have fillers, you have promoters, all those things, which help the product dissolve in your body and act in your body in the right way.

So, let me just simplify this in the sense of what is a generic for you. So, you have a tablet you take and it works for you. Right.

This is innovated by Pfizer. Now, Mankind or Dr. Reddy's want to make the generic and launch it in India. There are significant… Another question before you go on.

Yeah. Till the patent has run out, does Pfizer even during that time think that, okay, I can go to a Mankind or Dr. Reddy and they'll make it for cheaper. Why should I make it myself? See, the cost of production is not material during the patent time.

Right. So, they don't… 3%, 4%. So, they make it themselves.

But sometimes what happens, if a local company has more marketing muscles, they offer in license. Yes. We give you, we also promote and you also promote.

So, Mankind is very good in, say, some therapeutic area. They'll say, let's have them promote our product. Right.

Co-promotion, licensing, various models exist. But coming back to the product of what is a generic, you can develop the pill so that it releases in the same way as the innovator's product does. Right.

Within a margin of 20%. Right. So, if it's X milligrams has dissolved in your body over eight hours,

it should be within 20% of that eight hours and the concentration also should be within that 20%, plus or minus.

Another question. When I buy a tablet in the US, it could be paracetamol. When I buy a tablet in India, why do I feel like the US one works better? I don't think that should, that's subjective.

The irony basically is what, that 40-50% of drugs that you buy over there are manufactured here. Yeah. Indian companies.

But yes, I mean regulatory norms are very stringent when you export to US. I mean, one thing basically, which I know actually, when you basically make a drug, the regulator says that if the impurities are one or two percent, it's fine. For example, you say it is 100 milligram tablet.

So, it is actually the API purity is 98.9% or 99.5%. Correct? That means what is 0.5%? These are impurities. In the US, the regulator says it should be 99.8%. In India, it's all right. No, I wouldn't agree with that.

No, really? The standards are the same everywhere. How they are enforced are different. I think the standard of Indian regulators limits on impurities are perfectly fine.

In India side, you'll always find in tablets and capsules and medicines, overage is always there. Yeah. That means it is not 100, it is 100 some milligram.

Yeah. It is what actually? So that the excipient quantity should be that much. So that when potency loss is there, after six months, one year, two years, expiry date comes, it does not get below that particular level where the laboratory fails it.

Are the expiry dates shorter in the US? Like if something is not sold for a certain… There is a rigid science behind the life cycle of a product. And more or less anywhere in the world, we have the same standards. Like I take supplements, okay? Like I take everything from magnesium to sleeping to omega 3 and all of that.

If the bottle is six months old versus fresh, there is a difference? See, supplements are not well regulated. Right. So you're better off taking from a good brand who has good internal quality systems than rely on a regulatory system to do this.

But does it deteriorate in terms of efficacy inside a bottle? Every product degrades. Right. There's something called a stability program we run for every product we sell.

Every batch we sell, we retain a part of it to study how it's behaving over the life cycle. And if there is a problem, we recall it. Right.

That's a quality system all of us. But any drug you buy anywhere in the world, the expiry date is minimum 12 months. Minimum.

I'm just saying. So after that, it comes to a level when that quantity becomes less than what basically is being passed by the regulator. Is it right or wrong? See, it depends on the molecules.

In general, supplements don't have that rigidity in supervising and quality standards. But what form of… Question he has raised. Yeah.

I've heard the same question from a number of people. It's probably… Can I buy something from the US? No, you feel it. I'm telling you, you feel it.

Like… You should get it tested analytically and figure out what is the difference. Like, I always wonder like, why if it's the same thing, why should it be different? I'll give you one solution for this. Change the bottles.

I mean, nobody should tell you that this is an Indian or American. And then check. Ask somebody in your family.

Right. And then check. The blind test would always be drugs are same.

And see, I mean, here they're making in India. This is called basically, I mean, pharmacy of the world. Sells over there.

Good label. Nicely packed. Nicely kept.

In a right temperature. Right. Which is not here, actually.

So, plus some… Even today, in our mind, it's old, right? The foreign country… You may be right. There must be a certain brand you have that experience with. But as a rule, it should not happen.

Right. How big is the genetic pharma industry in India, size-wise? It is more than, I mean, 2 lakh rupees. 60 billion or something is what I heard.

We have our… 60 billion US? Yeah. Whatever question we want to ask, ChatGPT, he's going to Google it. Oh, he's got 60 there already.

Yeah, 60. 60 billion is 3 lakh. 60 into 90.

90, right? It's good for you. Why are you saying 90? No, no, it's not 60, actually. It's not 60, sorry.

The 60 has come down now because the money we get is in rupees. Right. So, when it was 80 or so, it was 60 billion.

Now, we have to discover… So, when you say 60, it is including export. Correct? No, it's Indian. Only Indian? Genetic pharma sold in India.

Yeah. It is less. Yeah.

It is… 30 billion is probably correct. So, let's say 60 billion total. 30 billion India.

30 billion India. Yeah. And 30 billion foreign exports.

Yeah. Yeah, that sounds… What pace is this industry growing at? Double digit or close to double digit. Right now.

The Indian industry is going faster. 8 to 10 percent right now, actually. Yeah.

And most of this generic pharma market share is between 30-40 companies? In the world? In India. In India, top 50 companies have a share of more than approximately 80 percent, 75-80 percent. Top 5 companies.

Top 50. Top 50. Top 10 companies would be having more than 40 percent.

And this is a 60 to 70 percent gross margin business? Must be, yeah. Indian market, yes. Indian market, yeah.

And the international market? It's molecule specific. The branded markets are more attractive. The commodity markets give you growth, value, inflection very fast, but the margins are lower.

And EBITDA is about 22 to 25 percent. 20 plus, yeah. So, the Indian market gives you close to 30 percent EBITDA.

So, you get more than that. That's because you add a brand to the product. Yeah, exactly.

See, I mean, it depends upon. I mean, if you just look at mankind, EBITDA is always in between 25 to 26 percent. Intentionally.

Because we just want to grow fast, spend more. We can increase it. I mean, bringing bottom line is a mechanism of what? Spending less, using certain things.

But we have never believed in that. We always said that if given a choice, we'll always go for top line growth. Right.

25 is fine. We're happy with 25, 26. Feels great.

But not compromise as far as the growth is concerned. Because what has made mankind? And when people say India is the pharmacy of the world, they mean by volume, they don't mean by revenue. Volume is what, 60 percent? 60 percent of the global.

But revenue is like 5, 6 percent. Not 5, I think 15 or so. By value.

Say approximately 5 percent or 5 percent. Why is that? Because we are in the generic industry. We are not the pharmacy of the world.

We are the generic pharmacy of the world. We sell generics largely. And there the pricing is much, much lower.

Over there, I mean, who makes the money? The pharmacies. Yeah. Yeah.

All grains of the world. So in the U.S., the innovator price, if it's 100, post patent expiry, if there are more than five players, the price falls to north of, the discount falls north of 95 percent. Can be sometimes 1 percent, 2 percent of the innovator's price.

So that's the value difference. We play in that part of the world, whereas they are on the premium pricing. When you're selling generic also, there's a brand attached to it.

Yeah. Even for generic. What our research was saying is, say, for example, Dr. Reddy's in many companies would have tie ups with doctors and specialists in tier one cities.

And mankind kind of penetrated the tier two and three cities. How big a part do the doctors play in the value chain of figuring out what pharmaceutical company do they buy their generic drugs from? In India, first of all, you're supposed to understand that, I mean, 95 percent products are branded generic. For example? 95 percent.

For example? Take any example. You take Bicasol, then there's a E2Z, there's a Fullhead, there's a 20 other brands. Methadaxe.

20 brands are there. You take Moxie and CBR or Argumentin or somebody else. 20 brands are there.

Right. The price points are different. Something is a bit 20 percent expensive.

Based on what? As per the company's philosophy and policy. So say, for example, mankind has a drug for, I don't know, headache. Okay.

And Dr. Reddy has a drug for headache. I'm a doctor in Goa. Now, how do I decide whether I prescribe your drug or his drug? Totally, I mean, it's basically relationship based.

Based on the salesman who goes over there. Right. Right.

Talks to him, convince him to prescribe. And it's all one-to-one relationship and the reach and the prices. And see, I mean… So these are pharmaceutical reps.

Absolutely. Which you both employ. Thousands of them.

Thousands of them? I think… We have 10,000. We have 10,000. We have 17,000.

17,000. When this mankind came, so our philosophy was that we will not follow any pharma company. Top-down approach will not be there.

Why should I go to bigger cities? I have no resources. Why should I fight in their battlegrounds where they are strongholds? We started bottom-up. Right.

When you go to a village and sell your product, you are supposed to understand the ecosystem, how much patient will carry in his pocket, how much doctor will charge, how much you can buy. Keeping everything in mind, we intentionally kept our prices 50%, 60%, 40% more affordable than the rest of the companies. So who gets what share? Say you sell a drug for 100 rupees in tier 2 town.

Okay. Tier 2. Same drug, I will sell in bigger city, same price. Same price? MRP.

Okay. Whatever MRP is there, if a villager is buying or a big city guy is buying, the price is the same. No difference.

So your retail price is 100. How much out of this 100 goes to say distribution margin to employ these 17,000? 10%. 10%.

No, no. Who employs what? Retailer. Who employs what? We employ 17,000.

You employ 17,000. Yeah. Out of this 100 rupees, how much? 100 rupees is what you are getting when the pharmaceutical guy sells it.

Yes. And you just take it like this, 100 rupees is the MRP. MRP.

Okay. That's a better way of looking at it. So in this value chain, how much goes to the pharmacy? 20%.

Okay. 10% wholesaler. 10% wholesaler.

Doctor? No, no, no. Distributor gets something. 2%.

But why does the doctor then pick one over the other? Relationship. What doctors? Relationship meaning you don't give money, but you give something else. No, it's like, for example, in our case, when we started mankind, nobody from bigger companies were going to these doctors.

Right. They were, I mean, nobody thought that one should visit them. Right.

We went over there, convinced them, we are affordable, we are good people. Right. They prescribed it.

I told you, FMCG. Why do you, tell me one thing, why do you buy Surf and not Nirma? Why is a watch being sold more in smaller towns? But we're almost talking, why does a doctor sell Nirma versus Surf? See, the point basically is what, doctor is a human being. So, I mean, somebody goes twice a month to you, wait for you, meet you, request you.

It's all about that only. But what is the balance? Okay, doctor, let's assume nothing. Pharmacy 20, wholesaler 10.

Distributor does what? Takes from you and… Yeah, 2-3%. He takes from you to the pharmacy. No, he does to the… Reverse.

I mean, 100, then 80, then 10% less is 72, then 2% less is wholesaler. See, this wholesaler delivers to the pharmacies. He has a group of pharmacies he'll send to.

Okay. This guy is a much larger territory. He'll keep the stock, take it here, and then this guy takes it here.

And distributor means 3-4%. See, distributor is like a depot or C&F. C&F.

Carrier and forwarding agent. Right. Distributor is that.

Stockist is wholesaler. I am a wholesaler. Right.

I get 10%. I supply to every pharmacy. Sometimes I give deliveries to the same pharmacy two times a day, three times a day.

I have my boys supply. Right. This is my job.

And distributor for doing this job gets how much? 10%. 10%. Wholesaler is distributor.

Wholesaler is distributor. Wholesaler, distributor, whatever you call it. Three things are there.

C&F is a… C&F, depot, super stock is whatever you call it. So, out of 100, you are left with 70. 68, 66 or something.

And then in the 60, you have to pay the salary of these 17,000 pharmaceutical reps. Their managers, their senior managers and everybody in marketing side, everything. How much does it cost you to make the drug? Depends upon.

I mean, for example, if I say that, if the gross margin is, say, 65%, so… 25. 25. 25 to 35 rupees.

Plus, don't forget that apart from this, there are certain schemes which every pharma company keeps running, actually. Like Apollo, I buy from Apollo. They have some points and all.

No. See, since the coming of online pharmacies, we started giving 10%, 5% kind of discounts, right? So, the same customer now goes to a medical store, right? And says, give me discount. So, he demands from the company.

How come? Right? Online has a different model. So, companies started giving 10% and 5% and 15% kind of, right, schemes to these medical stores. Funny story, when I was 17 years old, I

started a pharmacy.

Really? Yeah. I had two shops, one pharmacy, one laundry shop. So, you must be quite aware of what he says, yeah.

But I'm talking 22 years ago. Even 22 years back. I remember even back then, the highest margins were in some of your products, like condoms and stuff.

We made everybody happy. Yeah. I used to pay rent to be displayed.

I used to pay 10% of my shop's rent. My rent was some 10,000 rupees. Like I think 2,000 rupees used to come from displaying.

You've always been like that. Give to everybody and make everybody happy. See, I mean, point is what, you want to grow very fast.

You're supposed to really capture the imagination of the consumer, customer. How can you do that? Yeah. Who thought like that? In this, I have a question now.

Is online pharmacy a good business idea? I'm speaking up. It's still not, I think. Will it kill retail pharmacies like Amazon did? I'm not sure, but I think certain category of products will.

In this quick commerce, people are delivering medicines now. Exactly. Is that legal? Like they have a doctor call you up, 80 rupees doctor consultation, and then they send you the medicine.

Is that legal? I mean, this basically is curving the rule, actually. As per the law, without doctor's consent, prescription, you cannot sell. So, for example, if I go to online pharmacy and say that I need this.

So, they have a doctor who will talk to the patient, write the same medicine, sell it. Yeah, but I'll tell you why it's legal. Online consultation is legal.

So, what they are essentially doing is an online consultation. But Prashant, for example, I'm a doctor. I have written, you go to an online pharmacy.

So, if you have the prescription, just upload it. They'll give it to you. So, to our audience, you think, is this an opportunity? Is retail pharmacy or online pharmacy a business opportunity that's worthwhile? No? You need to have very deep pockets for that.

Just think, if you're selling online, why somebody will buy your product? Are you getting cheaper? Maybe I'm in their area and I can give them. The online thing is cheaper than a pharmacy. They give a discount.

I'm a nearby chemist. I can give you just one phone call away because these local chemists have become very active now. One phone call away, they'll supply the things, which was not there.

And they give a discount as well. But phone call is still an effort like quick commerce. You call a kirana shop also, he'll send you.

But people go on the app and press the buttons. Is it easier to pick 10 things than talking to somebody? Some people, I mean, at a different level, I mean, a small scale, have started this kind of apps. I'm not saying, I mean, like, for example, any startup.

They've started this. Tata 1MG, as we just know. They are big companies, but I'm talking about, I mean, at local level as well.

Small, Ram agencies have, for example, I'm just giving a name, hypothetical name. 50 chemists. And he has started his own.

And he is basically, I mean, spreading his app everywhere. Right. This is also happening.

But you're saying there's not really a big opportunity. Anywhere else in this value chain, distributor, wholesaler, are these businesses that 25-year-old can enter or probably not? They're all very mature businesses. Right.

Well established. Right. Difficult to disrupt them.

Right. So there's not much technology. See, if you basically… So online business can happen, but then you require a huge amount of capital.

It's not a 25-year-old's job. Tata 1MG is still not profitable. FarmEasy has raised 800 crores.

Yeah. Just think… You need that kind of capital. Think from a different angle.

Don't come in this particular line. For example, I'm a young guy. I mean, I know this technology, IT saving.

I start selling something else, which is not medicinal. For example, supplements. For example, vitamins.

But that's too crowded now. Absolutely. Everything is too crowded.

Till somebody breaks it, cracks it. How can they break it? For example, I mean, see, nowadays you must have seen all of a sudden, magnesium has become flavor. Right? 5X.

Everybody puts 5X, 10X, 20X. But somebody started first. I mean, somebody started dispersible tablets.

Because now cost is not the most important thing in this Gen Z crowd. I mean, for example, this youth. If somebody is concerned about his health, he wants a flavor, he wants a good looking tablet, capsules.

I mean, see, point is not bringing the same thing in the same packaging, in the same manner, in the same formulation. The point is making it very attractive in such a manner that you end up buying it. You become a brand.

Yeah, you become a brand. It's like that only. Like in a way, Boldcare kind of started from that.

That's the example. Are you interested in buying Boldcare? No. We have our own man force.

Right. We got many opportunities of buying them in different condom brands. Man force is man force.

Right. Nice name, I think. Man force.

My brother is very good in keeping names. Very good. Mankind.

He kept it. I'll say 80% names he has kept. Unwanted.

It plays a big part, no? Huge. The name itself. Very big.

Like Dr. Reddy, the doctor changed everything, I think. They have some very good names. Yeah.

Gomez. Yeah. Gomez is an amazing name.

Is there a science to it? Picking names? It should be easy. I mean, a four-letter word is always… There are not usual brand rules. Easy recollection, rolling of the tongue, recall value.

Right. They score a… On the basis of… And it should not be too close to the… On the basis of name of the molecule. Right.

Like this only. Right. Now, hospitals.

Where do they play a part in it? How many doctors are individual in India and how many are behind a hospital? I don't know the story, but I'd say most of them are individual. See, I mean, even hospitals, there are people who have practiced within the hospital. Right.

So, they have a control on what's described. But after the coming of Apollos and Max and Fortis and Nitralaya… Narayan. Narayan Nitralaya.

Right. Yeah. I'd say a good quantity of doctors are attached with different hospitals.

And there is a change in the mindset of the consumer and patient as well. That if I go to a single clinic and he says, no, go for eye checkup as well. I'm going for a fever or hypertension or diabetes, go for ENT.

So, instead of going over there, if I go to a multi-specialty hospital, I get one go all the things. Right. So, I feel today approximately 15-20% doctors would be attached with hospitals.

Yes, they practice separately as well, as Prasad said. You're saying 15-20% of doctors. And how does the hospital figure out which brand of drugs to sell? Again, meeting those doctors.

It's their call. It's pharmaceutical reps again. Yeah.

Yeah. Again. Even in a hospital.

Yeah, institutional. But they are different reps. They are a bit more smarter, more specialty oriented.

They probably get paid 10 lakhs a month. No, no. Nobody gets 10 lakhs a month.

To get 10 lakh rupees, what kind of a sales you're supposed to bring every month? I know. But if you have 22 months of expenditure margin, 1 crore. What is your first PCPM? Just curious.

Our PCPM right now is… Overall for 17,000. 7 lakh. Right.

So PCPM is per capita. Per medical rep. Right.

He's selling 7 lakhs a month. Each rep. Each rep.

On average. Average. Which is a good number.

Some would be 30 lakh. Some would be 50 lakh. Right.

But some would be very less. Right. But 7 lakhs is quite healthy.

Across the board. This is a digression. This is not related to your industries.

I work in private equity. A lot of my friends in the US especially are feeling very gung-ho about hospitals in India. They're buying many small hospitals, aggregating them, putting a brand behind them.

You've seen Max, Manipal's IPO is coming up. These are also friends of mine, the hospital owners. And they have multiples of 80, 90.

That's my question. Is that justified? If you buy a hospital individually, something in this locality, you're probably able to buy it at 5 to 10 times EBITDA. Yeah.

But once it's listed, it suddenly has a multiple of 80, 90 once a brand is attached. How do you see that business? To justify 80, 90, just calculate how much they have to grow at. Yeah.

The market is underserved, no doubt about it. But insurance is not well developed. The people coming in largely are under the government schemes, which puts a cap on earnings.

So for growth, you have to keep adding beds, adding beds, adding beds. And you see that.

Everybody says we are adding a thousand beds.

But they have this logic. They say in the US for every three people, there is a bed. In India, for every hundred people, there is a bed.

It's an underserved market. I agree. But it's like everything else.

Then somebody buys an unorganized or one of one hospital, which is not really running at the best level, excellent level. I mean, they will bring different processes. They will add some pharmacy.

They will negotiate better prices of the pharmacy with the companies. They'll negotiate better prices with whatever is being used in the OT. So they can bring some kind of excellence over there by bringing systems.

At the same time, once something has a brand attached, so validation is always there. So a big doctor can be brought, the doctor who has a name, can be visiting faculty as well. All sorts of things can be done.

And that's the flavor right now. Because you start controlling not only the pharmacy, doctors, everything you start controlling. Right.

At the same time, what happens, all the government schemes, which says that you're supposed to go for a one-day admission, then you get this. Right. They get the benefit of that as well.

So better, I'll say, better management. But do you think if I'm a stock investor, or I'm a private equity fund, do you think I should buy a hospital? What's your answer? You know it very well. People are buying.

Not people are buying, but what is your answer? I don't know. I'm conflicted. Because 80 times sounds bad, but these guys are growing at 20%.

But does 20% give you an 80 multiple? 20%? Consistently. Does? 20-25%? I don't know the equation, but I… See the multiple here is down from 80 to 64. I think it's a very difficult game to scale, because it's a real estate play at the end of the day.

Yeah. It's a huge amount of… The line he said is that India is underserved. It's absolutely correct.

I mean, if you just look at the difference between a hotel, right? Rooms, all sorts of things, kitchen, other things, versus a hospital. Right? How much money is there? Per room charges. And again, the point… Again, I forgot one point.

If I take a deluxe room, so my prices in OT, prices everywhere are different. I found that. I was in the hospital recently.

If you take the best room, you pay more. Double. Yeah.

Like from tablet to injection to everything is double. So those sort of things are there. The other thing is hospitals, the demand curve is not very elastic.

So if you have a doctor, you go back to him again and again. Yeah. If he changes his hospital, you'll go to him in another hospital.

But some hospitals have really changed this particular trend. Some people have done it. I mean, every time you cannot buy a new hospital and bring your best doctor.

Not possible. I mean, he can be a visiting doctor. If you take any chain, the hospital which delivers the maximum revenue per bed will be the one where the stars are.

The star doctors. Yeah. Or the founder is there.

So for example, you take Narayan Hrudayal, where Devi Shetty is there, you'll get the best rate. You'll take the max. Actually, they are not expensive.

Strangely enough, Narayana has to let 35 times. Not expensive, but that's where the revenue will be higher. Sam, just give me that mosquito bat.

There's one fly, I want to kill it. It's annoying. Flies are difficult to kill.

No, with that mosquito bat, I love doing it. It's a hobby of mine. Relatively lazy.

The fly is very fast. Those who go near the mosquitoes, they like it. It's almost like a recreational activity for me.

I'll go chasing mosquitoes and killing them. My son gets up at 3 o'clock in the night and starts catching mosquitoes. But killing them is satisfying.

So you're an animal lover? Animal lover. Okay, tell me this. Why is China First of all, before we go to China, we'll go to US tariff.

Tariff on generic drugs applies from April 26? No. They don't apply? No tariff on generic drugs. No, no tariff on.

Essential Commodity Act. Okay. So China and India are at par if they are selling in America.

So there are innovator products. If they're made in India, there's a 100% tariff. Really? Yeah.

So if you innovate a drug in India, you pay? No, if you're selling it to the US and it's an innovator drug, not a generic drug, it's not manufactured in the US, it's manufactured in India, there is a likelihood of a 100% tariff. So this affects our CDMO industry, the contract development

manufacturing. Yeah, I think we'll come to that CDMO.

That's also interesting. And government has started these pharmacies which are government- Yeah. Is that a big deal? It's in the long run, if it's going to erode the brand, anything, it's going to be for pharmaceutical industry.

It must have impacted a lot in smaller towns. Please. If really the growth is, say, 8-10%, had these Jan-Ujdi would not be there, the growth would have been much, much better.

Yeah? Yeah. How much are they selling, Jan-Ujdi? At a very low price. No, no.

Value? Value? Not so much. I feel, I mean, not more than, I mean, it's a rough kind of estimate, 4-5,000? 2,000. Even lesser than this.

The reason is there. But the value- When you sell something at a very cheap price, to run that Jan-Ujdi is a problem. Because look, if your daily sale is, say, for example, 2,000 rupees, what will you earn out of that? You need to have at least 50,000-1,00,000 rupees sales daily.

Then you earn 10%. In Jan-Ujdi, the profit margin for the person who's running is also very low. One, the MRP is low.

Second, you sell a huge quantity, but the value, per se, is less, and percentage to that guy is also less. The Jan-Ujdi stores are being run via capitalism. Is he looking to make a profit, or is it like a government scheme? The government is not running that only.

The government has given to somebody. Right. Somebody who is a businessman.

If a businessman is trying to make money from it. Per month, right? So per day is how much? 5,000. That's pointed.

So they won't be able to survive, you're saying. Quite difficult. And sometimes the complete, I'll say, variety is not there.

Every kind of medicine is not available. Supply chain issues are there. Also a bit socialist leaning, no? What do you make of that? If government starts capping the price of your drugs.

They are capping. The caps are there. They are capping.

Even though you're selling it a percentage of what it sells. Even then, they have a way of calculating what maximum we can make. And they give us inflation every year.

But there is a category of products which are listed as NLEM. National list of N-E-L-M. N-E-L-M.

Essential list. Published. These are all controlled prices.

Right. It's not a big list, is it? 300? Yeah. 350, 400 kind of essential drugs.

Paracetamol, Cifexem, Ziprofloxacin. The normal. I mean, a drug which is being used by everybody.

Most common. Right. National list of essential medicines.

We were reading somewhere that China is suddenly winning a lot of the generic contracts. Not generics, but innovation. Innovation? China's story is... What are they doing? Okay, there's a number two in the world in terms of innovation.

29% of all new drugs in cancer are coming from China. 70% of the biologics, novel biologics, ADCs, are coming from there. A very large number of the bi-specifics are coming from there.

They've overtaken everybody else except the US in innovative medicines. How? This is a long story. I think they started about 2015 or so.

But... I'll let you murder the... No, no. I'll fly first. So, China, as you know, they're a force in about eight or nine industries.

They've chose to become leaders. You'll never get a fly. Fly is not easy.

So, they did that in biotechnology. Yeah. So, there's a national fund, there's private, non-relative financing.

The stock market recognizes the value of innovation. There's a home market for innovation. So, they created an entire ecosystem for innovation.

How do they do that? I've seen them do this in so many industries. Yeah, so they have their playbook for this. I think they get talent back.

It's what they call a sea turtles program or something. We see venture capital funding. This is crazy.

Fly is not easy to kill. We'll keep it here. Let's see if the fly is smart enough to recognize the weapon.

That's your home. It's very irritating. No, my wife does this with mosquitoes, but fly is impossible.

Yeah, that's right. It's having a salad. Wow.

I'm kind of like starting to vouch for the fly by now. And they also have wrapped up research in their universities. So, the top universities publish a lot.

The largest number of scientific publications are coming from China now. And hence, they've built all elements of the ecosystem. Knowledge through universities and research institutions.

Risk capital through public-funded VCs. And, you know, government-funded private equity. I mean, they've created these funds for the private sector and the government collaborate and co-invest.

So, there's all kinds of incentives for entrepreneurs to really invest in innovation. What does India have to do to compete? Because this is new for them, right? Yeah. So, I would say the government has realized that this is a war they want to win.

And they said this explicitly in the last budget. So, they put some money aside. 10,000 crores fund.

I don't know what the fund is called. But there's a government fund 10,000 crores. Then there's an announcement that they will form a fund of funds which they will give to people like you to invest in companies.

They have a directly funding PRIP. Public Results for Innovation Fund. PRIP fund which they're funding innovation in companies.

Small amounts of money. What is it? Pardon me? Show it. Shakti.

Bioshakti. Yeah, yeah. That's the fund.

But there's also a larger fund 100,000 crores. So, it's a venture capital fund for startup. Yeah, yeah.

So, part of that will go to bio fund. And then they have something called the Research Innovation Program something. Which also they're giving direct money to companies for specific programs.

If you're successful you have to return one point some x. Otherwise, it's, you know, written off. So, lot of work is going on but nowhere close to the scale of these companies. The point is what? I mean, when you basically enter R&D kind of a domain.

Right. The chances of success are not like I mean, you're very sure that some kind of success would come. So, a company needs what? Or a scientist or a university need what? Grants.

Some kind of a grant. Some kind of ecosystem. Some kind of a help.

And help not for one year. It's not like, for example, now you have started and okay, fine. It will happen.

You need to consistently infuse I mean, some kind of a thrust over there. Money over there. Help over there.

And some kind of a support system that if you succeed you'll miss more. Like it happens in any startup. What a venture.

But even Trump is reducing this, right? Trump has reduced a lot of funding for the public research institutions. Yeah. Yeah.

Pharma here hasn't? For Pharma here they never gave any money. So they were funding public research in universities as well as there are you know National Institutes of Cancer and those were all doing a lot of research. NCI National Cancer Institute used to spend $40 billion every year in research.

Right. So the scale is very different. Right.

So China is the only country which moved so fast. Yeah. And they've overtaken Korea, Japan, Europe.

They're now next to US in terms of They do just a lot. With many industries. Yeah.

They are the people who are actually leveraging the capitalist system to its fullest. Yeah. Being a socialist and communist country.

No, they're exploiting the you know capitalist system much better than anybody else. Right. Do you think that has to do with the non-election non- bipartisan nature of governing the country where they can make a plan for 20 years? 100%.

Why not? I mean if you just plan something like R&D where I mean if you start today the chances of success are after 7 years. Right. In 5 years elections are there and some other government comes.

But Rajiv. I'm just giving you this is an excuse people say. If any state in our country has started a good program subsequent governments have followed through.

They've not disrupted it. Yeah. Very good policy.

You take our own state when it was a combined state Chandrababu Naidu was the master he put the plans together. There are certain states which are exceptional. I mean these are guys who have done good.

So if a politician does good it will make a difference. Look at the API market. Good politician bad politician.

Look at the API market or any other market wherever I mean for example if local industry is not being supported and some other country like China dump the goods. What happens? In any field it happens. So some kind of a support some kind of a barrier to entry is required.

Yeah but you know what I think we should learn from the Chinese. From? From the Chinese. These guys have achieved tremendous success in everything they went on.

I'm actually reading a book right now which is the biography in a way of Deng Xiaoping. It's about the transition of Mao to Deng. Mao was like cultural revolution fully fully communist and Deng came in and he kind of like converted it to some version of capitalism without calling it capitalism.

If I have to pick one lesson from what China did right is every state every mayor competed with the mayor of his neighboring state. It was a meritocracy. The mayor was in a progress.

Correct. The mayor of Shanghai you know goes to Beijing only if he does a great job in Shanghai. Same thing here.

I mean there are many ideas we can pick from this whole you know the way the Chinese have moved in the last 15 20 years. It's mind-boggling in every field not just our field. You take EVs you take.

I was there in China last January and I was totally amazed to see the kind of ecosystem they have built up. I mean I feel the day is not very far. He would agree with me that definitely will take US.

They are not competing with us. They are competing with the US. That sort of a speed and as a support and mindset and talent.

What he said in the starting that they brought talent here. But why talent would come from US? Why even leave? Because if you give that talent a chance to become an entrepreneur and government supports you in that. It doesn't happen.

I mean some kind of a support system is required. Some kind of a backing is required. Money, resources, and also I'll say that I am with you.

Yeah. I mean you're doing a good job for the country. When something of this sort happens, I mean these are really very impressive things.

I mean he carries that particular emotion in his heart. No, I'm a great admirer of what they have done. You see the quality of the stuff in our own industry.

25 years ago when we started seeing them come in with low cost API, low cost. I said, these guys, how do they make money? How will they survive? They'll go away from business. This was what we were saying.

Prasad, how are you seeing that API industry of India has been killed by China? They've not been killed by the way. I'm not going to buy that argument also. I think Indians still lead.

I'll tell you why we lead and how we lead. They're not playing the API game. They're selling these cheap intermediates to us.

We convert them into API. We make more money than them. So that, I don't think, is a problem.

The problem we saw is we didn't read them well. They were selling us this low cost material. We thought, oh, these guys won't survive.

The government is subsidizing. When the power prices go up, they will run out of business. But they were playing a long game.

They were just playing one by one. They didn't want to compete with us on the API, generic API or the generic business. They wanted to go to the highest end of the value chain, for which they put all the blocks.

They put a CDMO business, they put a chemicals business, they put all the petrochemical full integration from innovation. They own the whole chain. That's, I think, stupendous, if you see how they built it with such a vision and such a consistency, persistence, and long-range thinking.

They are smart people, actually. They are no less people. Aggressive, very strong, very fast.

I think because they have the upside. I feel like there's no fear, because at the end of the day, growth comes first, everything else is secondary. And B, the fact that they are competing amongst each other, not just money allocators or risk-takers, which are the entrepreneurs in society, but also the governments.

Each state government, each mayor, I think changes everything. Yeah. And, you know, if you look at the people in government.

So, you know, in our system, I don't know, the smartest people, the highest people are very smart. But here you see them, they're so sharp. You don't, you know, if you compare them with the private sector, some of our best brains go to private sector, they don't go to government, but here you see the best guys are in government.

Why is that? I think the system is very, you know… Because it's pseudo-communist, proximity to power is the highest value, it's not capital? I have no idea. I don't know, but I was impressed by the quality of thinking among the government people I met there. So, my experience over there in China was very different.

So, I met, I mean, a number of people, I mean, say, in the quantity of, say, 25 different companies, some startups. And in 50% of the cases, their background was US. Yeah.

Studied over there. 100%. In those universities.

Americans look like Chinese. They're Chinese. They were Chinese and they were called.

They came. What was the incentive? Incentive was that we'll just give you resources, ecosystem and we'll applaud you. You're a Chinese guy.

We're proud of you. They also have monetary incentives. A lot of monetary incentives.

And those people, those who have basically worked over there, worked under those scientists in Harvard and other Stanford universities, they come, they start. And if they succeed, I mean, initial money is being given. If they succeed, government gives them huge money.

Buys their product. Helps in every step. See, for example, I mean, if it's not only the resources and ecosystem.

If, for example, CMinister says, I mean, takes your name. This is Dr. Reddy, done fantastic job. Right? Call you on a stage, on a program.

Just a few lines. How much moral boosting it is for the whole science community. Yeah, but also… Give me some example.

I'm talking about the human side actually. But the human side is not the whole story. They're giving housing, which is as good as Western countries living quality of life.

They're giving… You're saying apart from everything, resources… You need everything. Just patting the back is not enough. That's what I said.

I mean, one point of program should be there. The next 15 years, 20 years, this has to really happen. Then we'll become some… See, which year China came in R&D side? You know it very well.

How many years? 2015. Yeah. Now? 10 years.

Now? Yeah. To give them full credit, they benefited a lot from the US. Not only just the talent, they also do all the big deals.

They don't develop product market globally apart from China. They get Pfizer, J&J, these companies to do deals within 50 billion or so last year deal making from China. And all the global trials are coming from China now.

So in a way, they're leveraging the US. I'll give one example, what I said. I'm not saying anything out of anything else, without having some kind of experience.

Recently, one of the scientists who joined from Belgium, he used to work in Johnson & Johnson. His name is Dr. Anil Kaul. Right? He was… It's a good recruitment.

I looked at it, the guy was very impressive. And see, this was a guy who basically was working in Belgium. Modiji went over there.

And in one of the seminars, he was there. He was working on infectious diseases. So Modiji said, you must come to India.

Right? And you're Indian, you must come to India. Can you believe? He left the job, come to India, had infectious disease department in India for three years. Wow.

I'm saying that. No, Modiji is very good, but Modiji is also one individual. I'm saying that.

How does it trickle down, lower down in the bureaucracy? I'm saying that. If the same kind of examples other big guys can take, it's never only the resources, never only, I mean, I give you resources, I give you everything, then I don't ask you. I need to ask you.

If I come to you and take your name and say what is the progress, the big guy is asking, something happens. It's a combination of things. You only seek validation.

I agree with you and I would build on top of that. I feel like you have to eliminate fear. India has too much fear in entrepreneurship that everybody is worried about this agency, that tax, this red tape.

That has to structurally go away. You have to be like, okay, net, net, is this helping or is this hurting? Sure, there are bad actors who should be punished and all of that, but the question should be net, net, are they inculcating fear or are they encouraging industry to do more? That is what I'm saying. I mean, If somebody has done a innocent mistake, understand, don't go by the, I'll say, the lines.

See the intentions and we all understand all these things. Smart people know it very well. We are planning to go to China in June.

We are hoping to do what China did to somebody in India, to China, let's see. It's an interesting place. It blows you away.

I mean, on one side, we compete with China, but certain qualities of China must be learned, brought it here. What's wrong in it? No, you should learn. It's always good to learn.

I was surprised we licensed a molecule from a Chinese company, launched it. And you paid a huge money for that. Not a very huge amount of money.

But you know, it's already a hundred crore brand in this year, the year of launch. We could never achieve such results. A lot of Indian companies are going over there and doing a licensing deal with those Chinese companies.

And they are very aggressive. You know, one thing that really surprised me, I'm moving from generic to an ancillary thing is Ayush and Ayurveda. How is it so big? Is it 30 billion? No.

How can it be that big? There are certain companies. I think this includes the doctors. It includes the people giving Ayurvedic massages.

It includes Live52. No, no. This also includes only, I mean, unbranded Jadibooties as well.

Like for example, other companies sell, lot of companies are there, plenty of things are there. Chilajit. And this is growing by what percentage? Live52 is a very popular one.

I remember because my dad used to take it. Liver extract. It's a liver extract, actually.

Like back in the day, like 20 years ago, Live52 was everywhere, that Himalayas Live52. My grandmother used to drink the syrup. I used to think there was alcohol in it or something.

Why is it so big? Why is it 30 billion? What are we missing? It's as big as the generic pharma industry of India, pharmaceutical industry of India. But if you just take the pharmaceuticals versus the Ayurvedic pharmaceuticals, I don't think it's much smaller. You're basically talking about as a whole.

And it's growing at 17%. Yeah. Yeah, as a whole.

It's growing 7% faster than pharma. I tell you, honestly, you'll also find a lot of growth you'll find in supplements nowadays. A lot of growth you'll find in vitamins and minerals.

Why? Because after COVID, the mindset of people have really changed. Whenever something allopathic is there, a pharmaceutical is there, it is to cure. When you want a longer kind of immunity, longer kind of, for example, I'm allergic to this, I get cold, then I switch towards these things.

So they become a kind of lifestyle kind of a drug for me. I'm taking Live52. I'm taking it.

All right. Yeah, I think the right comparison would be to not compare them with pharmaceuticals, but compare them with wellness products. Right.

A lot of them, you know, wellness, you take everything, Live52, you take various things like that. I'll actually add longevity and wellness under this. Yeah.

These are all preventative. Yeah. Right? Yeah.

Well, pharma is curative. Yeah. There are some things like vitamins which come in as supplements.

But do you think this is where the opportunity is for someone young starting a business? Good idea. Because it's growing at 17%? It is. It's a place to come.

But what should they build in this? What should they build basically is what, I mean, if you just look at the Ayurveda side. A, do you believe in it? I don't. You don't? I don't because as a company, we have committed to evidence, evidence back.

Like turmeric and ginger. We've done trials with turmeric. We didn't see significant.

Ginger. Yeah. Those are maybe preventive, but anything as a medicine, I don't know.

We don't. No, I'm not saying they work as a medicine, but these guys are claiming that preventing the disease is more important than treating the disease. So is there any controlled trial which shows that this is better than not taking it? Turmeric knowledge should be there, no? It's all, you know, anecdotal.

Yeah? Yeah. There's no control. So we sell and we believe.

We sell and we believe actually. What are the products you sell? Shilajit, Ashwagandha and Uterine tonics, right? There are Uterine tonics for liver. Lot of things we sell.

But what would you say when he says there's not enough evidence? I'm saying not enough evidence to back the claims that are being made. I mean, see, there's anecdotal evidence. There's history.

There is mythology. all of that is there. The original medicines came from plants only, right? Yeah, yeah.

They came from plants. And I'm not saying they don't work. But I cannot commit to that saying that, you know, this is backed by evidence.

So my argument basically is what, I mean, if we go to a clinician and if he uses it and says that it is defective, then only we'll use it again. And these are, I mean, time tested. We are not bringing something novel, something new.

These are being, I mean, time tested old medicines. Right. The problem basically lies in Ayurveda is that there are less, I mean, it's not very clear testing methods.

The purity is an issue actually. And a lot of time and regulatory is not very, very strong. There are complaints of, I'll say, adding certain other things in that.

For example, in this, in some of the drugs, lead has been found. Heavy metals. Heavy metals have been found actually.

If somebody brings a very pure kind of a thing. Stabilization, standards. I mean, the right standards really happen.

When you say Ayurveda, right, the purest form of Ayurveda is being exported. Yeah. See, what is basically Ayurveda? The leaves and branches and trees, whatever is there.

So, what is the most pure form versus the rest of the material left? So, when you, from the rest of the material, you bring the extract and sell it on the name of Ayurveda. Right? It's like tea, the

best stuff goes out. Yeah.

Same like that actually. But why is that? If you, in India, all these tiny brands which are coming up, Ashwagandha, Shilajit and all, they charge 3000 rupees a bottle. We sell like that.

Yeah, then why not keep the best stuff? The point is what? There is a big market. Look at the big market. Look at the price they get, the profit they get.

I mean, always, things follow money actually. You're saying there isn't a big enough market of people paying 3000 rupees for a bottle of Shilajit. That's why it gets exported.

I mean, everybody says, it's a Shilajit, it's a commodity. But somebody, I mean, put a brand on it, give a right marketing and says that we do it like this, only in the purest form. It comes from the Himalaya.

I mean, you need to market it very, very well. But you're getting the export stuff or you're getting the leftovers? We are an 85% India company. So our focus has always been India.

So whenever we export something to conditions, it has to have some kind of a, I'll say, difficulty in manufacturing for others. It should not be a vanilla generic product. We don't want revenue from that place.

We want that longevity. We want profits over there. We want some kind of respect from that side.

And that's one reason we have chosen this path. Do not sell churnal products. We don't want top line from that side.

If some complex product is there, difficult to manufacture product is there, But you're saying this 17% is right? How big of mankind's business is this part? Minuscule. Going crazy? Yeah, going good. Not crazy.

In Ayurveda, I mean, it's a slowly accepted kind of a product. It's been around for centuries. You're selling something at 3000 rupees.

So how many people would be buying 3000 rupees Shilajit? Premium class, no? Like 10 million people in India? 10-20 million? Even less than that. And out of those, how many will be convinced? And then you basically send these. But what we basically do, we send these products.

We want a paper from Ayurvedic colleges. You try over there. You give us some kind of a paper.

And then we'll sell it. It's a long process. Government was planning to make Ayush doctors like normal doctors or something like that.

Right? It's a topic I don't want to talk about. No? Do you have a view on it? Yeah. Good.

Okay. Why not? If a chemist can give medicine to a patient, why not an Ayurvedic doctor? He has some kind of a knowledge. Plus, they can be educated as well through online courses.

And many doctors are ready to give this kind of education. So, when I used to work, in those days, BIMS, doctors, every kind of a doctor used to practice. And they were what? They were Ayurvedic.

And sometimes... I mean, my only concern is if it is structured, standardized, based on evidence, based on facts, it's great. You know, a lot of this, my father is an Ayurvedic doctor. I become an Ayurvedic doctor.

My son becomes an Ayurvedic doctor. That to me is something I don't think is logical and good. See, I mean, I tell you, Prasad, there is a... If there is a... I mean, if there is an education system and it has standards... They have Ayush College, no? In Goa only they have one, big one.

I tell you, in this world, there are many worlds. In pharma world, there are many worlds. There is a world of cardiologists.

There is a world of... No, for me, it is great. For health, wellness, it is great. Because there is good in all this.

The herbs we eat, all that. But, I think it should be treated like that. It cannot be said that Ayurveda is better.

Let us do this. That kind of thing. So, everybody knows it.

Even Ayurvedic doctor knows it. If a patient is coming to him and if he prescribes only Ayurveda product, the patient will not come back to him. Because patient needs what? Instant relief.

Faster relief. Some element of Ayurveda can be mixed into our traditional curriculum for wellness. Yeah.

For wellness, preventing. Wellness is not a topic at all in today's medicine. But somebody, just think somebody who is just practicing.

He wants to earn money as well. The money will come through patients only. Why you will? I mean, see, we all want to have money to run our life.

Doctor, it is a very noble profession. But he, teacher is a very noble profession. But why not a teacher should be given a better salary? And soon, maybe anyways with AI, all doctors will be a computer.

That's the point. See, if a chemist can substitute the product, if a chemist can say, give me cough medicine, he will give it to you. Ayurveda will be better than Rotos.

But is that the system we want to build for our country? It's happening with pilots also. Now they are saying, pilots don't need to go to like actual plane. You can go to a simulation.

No, but you know, there AI makes sense. Because you have safety. He will not agree.

But it's safer than me driving. I tell you, a scientist will not come to this side. Because I come from that side.

I have seen the worst of the world. No, no. If the science is good, we should support it.

No, no. If the logic is good, we should support it. Sometimes, without logic, practical knowledge of a person, the patients he has seen, the people I have seen, and treated and cured.

Sometimes, you will find in small villages, there is an Ayurvedic doctor, he is using a very, that particular area's medicines. Full respect to that. I mean, if he has knowledge, wisdom, we should use it.

But as a system, does it scale? Does it? Right. But if this is growing, I am saying Ayurvedic doctors. So, I would suggest that the opportunity here is wellness.

Yeah. Like you said, longevity to it. So, around wellness, it can be tourism.

It can be many things. It can be supplementation. It can be the, you know, yoga, workout, all these things.

Biggest in Ayurveda would be prevention side or immunity side. Okay. But before we leave this topic, we came to the conclusion that Ayush is growing at 17%.

By Ayush, I mean doctors, Ayurveda, Shilajit, Live52, massages, longevity, wellness, sexual wellness. Yeah. Great market.

Supermarket. Best market. Where Shilajit sells? Actually, it's the same thing.

Where it sells? Isn't Ashwagandha for stress? Stress. It's for stress. Stress, no? Aphrodisiacs are required, it is being used as that.

What, what percentage of sexual wellness is growing at? Tremendous. Yeah. Really? Yeah.

Maybe in this 17%, 30% would be even sexual wellness. you have a brand, right? Yeah. You have a Viagra, I mean, Manforce, that's a big seller.

Is it Viagra, Cialis? Yeah, Viagra. Boldcare has scaled to a certain point and has slowed down. Why is that? No, they basically are in the process of selling the company.

Right. Right? When you sell, what do you do? Sell. Spending is less.

You are supposed to show profits. Right. That's the reason for that.

I am an investor in Boldcare. Hmm? I am an investor in Boldcare. So you must be knowing it.

You must know. The ad was what worked for them. I think that Tanmay Bhatt did a spectacular job of shooting that ad with Ranbir.

Marketing is so much. Marketing? Right? But again, I mean, I tell you honestly. I like your boldness and how you I tell you, one more thing.

We can't do that. I tell you, very inward looking that way. See, one more thing is there.

Marketing does not mean that you pick certain USPs. Unique Selling Points or certain special points of your product and project it, amplify it and give it to the people. It also requires right from formulation to packing to flavor to taste to doses and then make it so good when you market it.

I say, Nikhil, this is a great product. This is marketing. And I acquire you as a customer.

You buy it. But when you buy it and you use it, you say, okay. On the second side, you say, amazing.

Then I'm not only acquiring a consumer, I'm getting a loyal customer who will just market my product to other people as well. So, it is actually in the hand apart from marketing, what basically matters is what is your product. You market a bad product, maybe it will sell.

Right. But, longer run, it will fizzle out. I'll ask you another question.

Like, sexual wellness products like condoms and Viagra and all of that, there is a societal stigma that people feel shy. It's also in some ways an Indian thing. I mean, if I've ever tried to buy a condom, it must have been a terrifying experience for me to go to a pharmaceutical store because we're just like that.

You know, like, if you're buying sanitary pads when you're a kid for your mother or you're buying condoms, do you think there is an opportunity for a business which is online only, sexual wellness, all these products? Yes. Yeah? Yes. And if you just look at this right now, there is approximately 50-60% growth of condom sales online.

Online is just on the fire. Condom sales, pregnancy card sales, people want in just 10 minutes. But then for most people who live with their families, even when the delivery happens, isn't it a worry that my mother will open, somebody else will open the box? Quick commerce, no? 10 minutes.

It comes by the time you put your phone down. You are present at that particular time. Right.

True. You're selling Rye Kennedy. So actually you're right.

Huge. Even these pharmaceutical products, Blinket is selling them like crazy. Apart from this, just one more, ask one more question.

If I have some problems, sexual problems, will I go to a known doctor? Will I go to a chemist? Known chemist, no. I can talk to some unknown face online who can give me solution, I can follow it. Because he does not know me, I don't know him.

Right. I just want advice which is coming from an unknown personality. So there is a, I mean, secrecy.

Yeah. Because even when a patient goes to a doctor, the doctor tells him something that this is your problem. He comes back with these kind of feelings that in front of his doctor, I've lost my, I'll say, respect or whatever you call it.

So whenever he goes with his family or wife, he feels a bit let down. Yeah. I was seeing the numbers of fertility businesses like people trying to conceive a baby.

Even that is growing like mad. It's going much crazier. We've had one company, Bharat BSV, that company is specialist in this one.

Fertility. IVF. IVF, yeah.

And even like women who want to have children later in life, that seed, what is that called, egg, freezing eggs, that's also growing like crazy. I have so many friends of mine who are getting married later in life and priorities. It's all career priorities.

There's a reason for that. Education, urbanization, women coming in jobs, late marriages, career first, and I'll see kids later on. Yeah.

And at that particular time, I mean after 30, that, I mean, for, body is not that much fertile. With every passing year, after 30, 20%, 30% it is less fertile. Yeah.

That's the problem. Another problem basically is what? There are number of days in a month which is more fertile. So I was just talking, so why don't people really go on those seven days? Some doctor said to me, how are you sure that on that particular moment, on that moment? So now, she's most fertile or this sperm and ova will just fuse together.

So now people are doing what? They're taking a short honeymoon on those seven days for three days. And how do you figure this out through medication? Yeah, the kids are there. You make those kids as well.

Right. So if somebody does, you can't add sexual wellness with this though, right? With which date, what and all of that? Which date she's ovulating? You can add all of them in one business? Why not? And it should not be called something sexual. Then the stigma is lesser.

that's important. Look at this, how this company is promoting their brands, how this any sex, your company, how they promote them. Yeah.

But it is like that. I mean, net-net, I think the way the world is going, I might not have children, but the world needs children because too many are not having any, I guess. I don't know about India, but the world needs more children.

India too, actually, when you look at the average age, I think it's gone up from 24 to like almost 30. Soon, if we don't have kids, I think that fertility ratio has dropped below 2.1, which is a replenishment ratio in most states outside of Bihar and UP or something. People are not having two kids.

Any state which is more educated, it is less. I mean, more education is leading to less number of kids because they feel that to give education, the right kind of schooling, so much of money is required. You go in West Bengal, eastern side, one kid.

You go to this Kerala. Can we do another digression? I want to take a science lesson on what is API, what is small molecule, what is big molecule. You'll have to tell me what all is there.

Sure. Start with API, active pharmaceutical ingredient. That's an organic compound mostly, but sometimes inorganics can also be there, which is actually the part of the medicine that is active in your body.

That's why it's called active pharmaceutical ingredient. When people say, I don't know, I have zero knowledge, idiot. When people say API industry is controlled by China, India doesn't have it, what do they mean? I don't agree with them.

I think the Chinese are much more low cost than India due to variety of reasons, but on the front end, they're not that good. But can we make API without China? Today, we could do it. Right.

We have all the capabilities to do it, but the economics dictate that I buy and I don't make it. This is what I was saying. The economics, it's easier for me to buy an intermediate.

But if tomorrow China stops supplying? We can make it. Yeah? Most technologies exist in India. At scale? At scale.

As required? We used to make them before the Chinese were there. I don't see the technology as a limitation. China brought the prices down because we could not protect.

For example, look at this intermediates, right? China does not give. For example, take the example of antibiotics only. A lot of antibiotics are there.

In the absence of Chinese supply, we can come into a crisis. India will make it. They're already making it now.

At that level? At that level? Yeah. So we were making these penicillins before China was making them. So I don't think it's a question of capability.

It's just a question of our focus. The question basically is what the question should be asked is why did we stop making? Because it's cheaper to buy. Correct.

Yeah. And I would not criticize that. There is no need of national security on this.

It's availability, you buy it. If the availability is not there, you make it. But now this is national security.

Pardon me? Now it is national security. I don't agree with that. I mean, that's a point of view always.

But I feel we are pursuing the value chain. We are capturing more value from these products. We don't see the Chinese competitors in the finished products anywhere, including emerging markets where they should be strong.

We are selling in China. It's a global world. Why are we saying in national security supply? No more global world now.

In COVID time, I'm telling you this, in COVID time, we had more reliable supplies from China than from Indian companies. So what is this thing about national security, API controlling by them? This is all a narrative I don't believe in. Do you believe it in batteries and energy and all of it? Maybe, because there is supply there.

There is a source of raw materials which we may not have. Right. And that is strategic and they have threatened to control it.

But in API, there is no control, there is no threatening, it's all available. And if China stops tomorrow, we can… Why would China stop tomorrow? Let me ask you that question. Like China threatened US on rare earth, same logic.

Yeah, but there is still supply. But for a while, it was negotiating. I mean, that's common in global politics.

If you bully and you want to be the only bully in town, we will be bullied back. So, that's what happened to the US. It's like saying food security.

Like, should we import all of our fertilizers or should we have capacity? But we are not importing all the pharmaceuticals that we need from China. That is not a narrative. APIs? No.

API, we make a lot of APIs. India is a big exporter of APIs. In fact, the competition is severe among Indian companies.

It's not the Chinese which are competing with us. So, where does this come from? The Chinese are in a different league actually. Chinese want to basically have huge profits.

So, why should they spend their lot of energy in this kind of… They will sell biologics. But yes, it's a national security issue now. Even biologics, I'll tell you this, they need us to sell the product.

They need Indian companies or western companies, more western companies than Indian companies, to sell the product. But tell me this as a very, very layman, like somebody who knows nothing. When you say organic compound, I'm still confused.

Okay, it's like you take simple molecules like starting materials which are called basic organic chemicals. Okay. Then you react them to get a more complex compound.

That's an intermediate. Then you combine some intermediates to get the final product. Okay.

That is the API. The final product is the API. API, yeah.

And the API has to be at a very high purity level. It should have impurities below threshold. I know eating paracetamol.

Where does the API come before? Okay, paracetamol. It goes into my mouth. Paracetamol is the API.

Okay. The tablet you're using it contains 500 mg of paracetamol and a few other binders and other things which make it deliver the paracetamol in your body at the right place. Okay.

So that's the formulation. Formulation is what we talked about so far. The pharmaceutical industry.

For example, when you take a paracetamol tablet, the weight is not 500 mg. The weight is more than that. It's the excipients.

Inactives. Inactive. Active is the paracetamol.

Paracetamol actually chemically is para-amino-phenol reacting with acetic anhydride. Paracetamol is a short form of it. Moxisline, Cephexine, Ciprofloxacin.

I'll give you the synthesis of paracetamol. I don't know if it's going to be accurate. You start with something like nitrobenzene or something.

Reduce it to make amino-phenol and then you acetylate it. That becomes paracetamol. Right.

So the three, four reactions involved. Finally you purify it and formulate it. So the powder that we sell as a chemical is paracetamol.

And that's API? That is API. Okay. So like that you have Ibuprofen, you have Abicillin.

Those are all APIs. The active ingredients. Now, biologic product is much bigger.

After API, biologics. In terms of ingredients. These are synthetic.

API synthetic. Yeah. Synthesis.

We call it synthesis. Organic synthesis. These are smaller molecules.

Biologics. No, the APIs that you're talking about. They are smaller molecules.

They're called small molecules, bulk drugs, APIs. All these various terms are used. Raw materials.

So basically that is a molecular weight will be at most 1,000, 2,000 is the highest limit. Around 1,000 is the thing. After a certain size they become large.

And they're synthesized by living cells. So what you do is you develop specialized cells which make the ingredient that you want. Biological systems.

Synthesis is like Lego works. You're attaching molecules. So API is a Lego block.

Yeah. And under 2,000 whatever. You Lego block 2, 3, 4 smaller Lego things, you get a bigger one.

But its size is 4. 4 blocks. Just for example. Yeah.

4, 5, 10, whatever. 1,000, 2,000. Yeah.

Whatever. But here it's a living body, living cell, living cell. In biologics.

In biologics. Which you inject a clone to produce a certain product. And it could be as simple, there are different kinds of cells to make the it's more complicated.

interesting. When you say inject a clone, what do you mean? This is interesting. a clone, what do you mean? You take a sample of DNA and you modify it, it's called recombinant DNA.

Okay. To tell your cells to produce this. Right.

Which is your, I am done. Try some. That's a lot of food, I can't have this.

No, just try a little bit. And you eat it. And then the cell, you grow the cell, you multiply the cells, you feed it glucose or whatever to multiply the cells.

Cell is like a real living thing. Yeah, it's a living unit. Where does that come from? There are various kinds of cells, mammalian cells which are complicated.

You can use yeast cells, which is a high yield, low cost. And then you used, what are the other kinds of cells, insect cells, plant cells. All of this produce something.

We are actually producing many things as we live our lives. So in biologics you are taking a cell and putting a large number of API together which is more than 2,000. No, no, no, sorry.

This is different. This is over. Remove that from your mind.

Now you are making, these are chemical drugs, these are biologic drugs. Biologic drugs are larger in molecular weight and they are produced through a variety of means. I am not a biologist so if I am saying something inaccurate, forgive me for that.

But in very broad terms, these are drugs produced by living cells. And cells can be many types of cells. Okay.

Insect cells, mammalian cells, yeast cells. Okay. They, you modify the cell by putting in a clone of DNA into it.

Okay. And it multiplies. Okay.

So you grow some cells, you grow them in a larger vessel and you take them to production scale. Okay. You have what you multiply them and then you multiply them to a certain size and then it's produced.

The cells produce this. Then you have to kill the cells. Right.

After they grow for a while and you get the right product produced in the cell, you destroy the cells. You actually break the cells and extract out the product that you want. Right.

From the cells. This is broadly, I mean it's not very scientifically accurate what I'm saying but this is the broad process. What are some examples of biologic? So he talked about ampicillin.

The penicillin there is a biologic drug actually though it is not called biologic. What is ampicillin used for? It's an antibiotic, broad spectrum antibiotic. So antibiotics come under biologics.

No, not every. Antibiotics are synthetic as well. They are actually semi-synthetic.

You take a biologic, you combine it with a chemical, you get a semi-synthetic. So that's a complicated thing. Don't just take that out.

Your GLP-1s are peptides. They are larger molecules. Now they are not biologics.

They can be done as biologic, they can also be done as synthetic. But synthetic is very cumbersome, you have to do a long synthesis. And that's a class of compounds called peptides which are made out of amino acids.

You can also tell where these biologics are used actually. Autoimmune diseases, cancer. Many diseases use biologics from simple diseases to complex diseases.

He's talking about the targeted biologics. They are a class of compounds called monoclonal antibodies. These are like, they call them the silver bullets.

They go right to the place where your body needs them and latch on to the target. So they're more precise, they're more accurate, they're more effective for many diseases, especially cancer, immunology. They're all biologics.

Where do vaccines and all come from? Vaccine is a biologic. It's made from a virus. You're actually taking a virus, immobilizing it so that it doesn't cause you damage.

You're putting it into your body. Your body recognizes the virus, develops antibodies. Now tell me, what are stem cells, peptides? I recently got a lot of stem cells.

And the doctor told me it's from some umbilical cord or something. Stem cells are a special kind of cells. They can take, they can become any part of your body.

They can become muscle, they can become bone, they become skin, they can become your organs. So they're like magic. And when they say they're from donor, embryo, umbilical cord, what do they mean? It means stem cells are not everywhere.

We won't have stem cells. Stem cells are when newly born children, they have them in the umbilical cord. The mother produces the stem cells by which this body, nature is produced.

So the stem cells are there in the cord that is attached to the mother and between the mother and the child. So that is stem cell rich. So they harvest stem cells.

And you get unlimited stem cells from one umbilical cord? No, no. You can multiply the stem cells once you isolate them. You can grow them.

The origin is the umbilical cord. From there you can multiply it. So my question is, one umbilical cord can have enough stem cells because of multiplication for thousands of people? There must be some limit to that because they get tired and they don't reproduce.

They don't divide so much. And peptides come under stem cells? No. Peptides are not cells at all.

Are stem cells good? Stem cells are cells. They are not good or bad. Is there science to say they are good for us? They are being used for therapies.

Should I be taking them? They are not FDA approved if you are asking that question. I'm not asking that question. Should I take them or not? It depends on what your problem is.

No problem. Then you shouldn't take them. No? No.

Really? Yeah. Why? Lack of evidence. They just do something largely originating in Korea, China.

It's business. In Dubai also? Yeah. But if you do research on the websites of the FDA, you will get sound advice.

Not to do it. So some people, if you have a knee problem, cartilage is damaged. The stem cells can grow the cartilage is what they are saying.

But again it's still not medically proven. Cartilage because I play a lot. In my knees and I take all this.

I'm asking actually for myself. You're advising I'm actually taking them. Has it helped you? I don't know.

These are all. I get tennis elbow. I don't know if it helped or not.

And I get this sound in my right knee which is like tuk tuk tuk tuk tuk. And they'll screen you for everything. And they give you sound medical advice.

But is there like if I there's a lot of hype around peptides. I'm hearing it for the first time. I've seen getting injected in their face and all.

I wouldn't do Dubai is a place. Dubai is a place. Collagen will be boosted.

Collagen maybe. These are all non-medical issues. So peptides are a class of compounds.

Your body has peptides. Okay. Peptides are smaller than proteins.

They're made of amino acids. Proteins are also made out of amino acids. These are smaller in chain.

30, 40 amino acids. 50, 60, 70 more. Proteins will have hundreds of them.

Thousands of them maybe. So it's a class of compounds which are becoming large part of the therapeutical basket. So semaglutide is a peptide.

Terzapatide which is Monjaro is also a peptide. This is a big one. Semaglutide, what is the story? It's like the whole world has changed.

I have heard 10 friends of mine who want to start business in GLP-1. What is it? So they'll start the business in GLP-1. What do they want to do? Do they want to sell this product? It's a prescription product.

They'll build a marketing around it and create a brand. Forget the business. Forget the

prescription.

How many people are using it versus how many people are abusing it? So whenever a product becomes super hit, right, it is being abused. I want. If a patient forces a doctor, he says, no, I have to get it done, then it really happens.

But just imagine. I think you asked a very question, where did this come from? This is a good question to ask. This is a product which your body produces, actually.

GLP1, not glucagon-like peptide, but a peptide called glucagon is produced in your gut. It creates satiety in you. And the only thing is it is not stable.

If I just take that and inject it into you, it is not a stable molecule. So, doing a lot of research, wanted to extend the half-life of this product and they made some changes to that peptide, which became semaglutide. Your body has multiple hormones, which trigger your brain activity to say stop eating or your satiety is very high, you can't eat, or if it is deficient, you eat and then become overweight.

So, if you control the level of these chemicals, your craving for food comes down. And then you don't feel good. In the head or in the gut? It is head and gut, there is a connection, I don't understand that.

A lot of people say that. Yeah, but it is largely gut. But they are circulating in your body, in the blood and all that, so there is a very good chance they are going there also.

And when you say no or not is figured out how to increase the half-life, it still disintegrates, but longer. So, whatever the issue was in the beginning is still there, but it comes after a while? No, not that way, not that way. Every chemical has to go, it can't stay forever in your body.

So, the half-life… Even if your body creates it? Even if your body creates it, your body is circulating a lot of things, metabolizing them, all of that. So, your body produces chemicals on demand, antibodies on demand, all of this. So, it is a continuous system, very complicated, complex system.

So, when the GLP-1s, they figured out that this hormone has a role in hunger, triggering hunger and all that. So, but they couldn't find a stable formulation or a stable keep the product long enough in your system to make an impact. That they figured out through the use of semaglutide.

That's how semaglutide came. Around the same time, Lilly also worked on this, developed their Mungero and it took them, I think… Are you for it, against it? I'm for it because it helps a lot. Not for maybe you and me, but if you're clinically obese, you have hypertension, you have cardiovascular risk, you have metabolic syndrome, you have diabetes and in some cases they are now saying even Parkinson's.

So, if you lower your weight, there's so much risk which comes down. Of course, it is not easy to… Especially metabolic disorders, diabetes, diabetologists, they can prescribe I think it's a great product. But, you know, it's not for making a model to be slimmer.

But where the use is happening, newly brides or those who basically are about to get married, I mean, anybody who wants to lose weight without doing anything there, the problem is you also lose muscle. Muscle, yeah. That's a problem.

It's not just fat that you lose. You lose muscle. And once you lose muscle, they don't come back, no? They come back, but you have to do a lot.

You have to increase your protein intake. You have to work out all this. So, along with the prescription, there's a list of things that the doctor should be telling you.

So, we have launched this semaglutide with this philosophy. Our philosophy is totally different. Number one, talk about the side effects, contraindication, just to get the idea.

All of us talk about it. It's not like we don't talk about it. We are ethically bound to talk about it.

Prasad, the point is what? We start with this. We don't end with this. Number one.

Number second, just remember, at the time of gold rush, who made the maximum money? Those who run for gold or who basically sell the ancillaries. Right. Correct? So, what our philosophy is? Along with this, our focus would be on supplements.

I mean, this would be front end, but actually, we will be selling the supplements, vitamins, other requirements, the complete chain, the ecosystem we have created for this particular division. That's our concept. And again, I tell you honestly, in pharma industry, nobody talks side effects.

One of my friends once told me… I beg to disagree on that. And you are painting in a very broad way of pharma industry. I think we all are bound by the ethics of running a prescription-driven business.

Please, don't take it personally. Not personally, I mean, you said industry, that's why I'm protecting that. If I start, have you ever seen a sales rep, because I have spent my life, right, talking first about the side effects versus effects, you'll not find one.

You'll say, yes, these… But you have to start with the disease you're trying to treat. It's not that… If I know, if I know 50 brands are there, 50 companies are promoting, I'll not promote that. I'll say, no, take precautions.

This is not a shortcut. I just want to come in the limelight through this. And then promote my other products.

In a way, it is called ethical selling. There is no ethical Pharmaceutical selling is not supposed to cross some boundaries. And we should not.

I think largely, the bigger companies, top 10 companies, don't cross that line. There are companies who will sell you anything, under the promise, but I don't believe that the top 10 companies do unethical selling. In India, I don't see it so much.

In the US, if I'm watching a TV news channel, half the ads are pharmaceutical drugs. Their direct consumer is allowed. Here, it's not allowed.

But they also say, consult your doctor. They say you'll have vomiting, nausea, headache, this, that. They say all of that, but they're still selling.

But you'll find that, how boldly they write, what letters they use, how much time they spend on explaining the side effects. You'll find that properly. They say 20 diseases but nobody pays attention.

They still buy. That's fine. That's okay.

But I'm telling you. But you know, in the US, you can't buy a drug just like that. You need a prescription.

Here, it's easier to buy. You can go in and buy ampicillin, amoxicillin from your corner pharmacy. In the US, try to get a medicine without pharmacy, a registered prescription with all the details.

So you both make JLP-1. Yeah. I just want to make an important point here.

I agree you have a point of view about marketing. But our industry is an ethical industry. By and large, 100% of the top 10 companies don't do anything wrong, is my view.

I have not seen this. I also go and talk to doctors. There are small companies, this is a very large industry, hundreds and thousands of companies are there who may be doing many things wrong.

But you have 10,000 reps. How do you know what each person is saying? We monitor them and if a doctor complains, we do get complaints. We get written complaints saying your med rep said this, let's call it.

If we know, we won't keep quiet. Okay, more importantly, what is the business opportunity here? You started with peptides and then went off into GLP-1s. Peptides are many things, not just GLP-1s.

There are many peptides and now, since you're into fitness, there's a whole wave of peptides being marketed to young people. Sleep, sleep protocol, GHK, CO, copper, peptide, I'm taking one or two. Consult your doctor.

I try everything. So peptides are a class of compounds which are becoming very important even in the pipeline. The pipeline has, and this is an important shift in the industry, pipeline used to be largely small molecules.

It's now shifting to large molecules in biologics and peptides and also another class of compounds called oligonucleotides. This industry, India is not great at biologics. In biologics or peptides? All of this.

In biologics, small molecules are the masters. We can compete with China, the world, any… The generics fall largely under this? Yeah. Generics are across the board.

They're called different things. Generics is a formulated version of the ingredients. Once something can be made by everybody, it's generic.

So APIs used to be the king of this thing. The value of the pipeline used to be 80-90%. The rest used to be vaccines and small biologic drugs like enzymes.

Now, because of the improved efficacy, the precision of how they work, this has picked up a lot. Biologic drugs. This is where India needs to pick up.

Indian scientific and manufacturing abilities are lagging. China is way ahead of us. I won't say that in chemistry, but I'll say that in biologics.

Kiran's company is very much in biologics. I don't know what the revenue is. The companies that are competing with her in China are massive now in the number of products they're developing.

Insulin is also a biologic drug. Originally insulin was extracted from pancreas, but now it is developed through living cells. So, this part is something the companies have to invest in.

This is also the part where there's disruptive science developing, technology is developing, and could be opportunities, but not for a general 25-year-old, but somebody who is deeper in technology, who has a background. Maybe Indian Institute of Science or IIT. Those are the guys who can think of this.

If 10 chemistry biology people get together, can they start a company in something here? They need a lot of capital. Which is not available in India. It's not easily available, but it's available.

They can do innovations and connect the dots to a larger company. In Bangalore, you find people doing interesting work. You can also generate novel biologics at a relatively lower intensity of cost than small molecules.

It's easier to develop. Can I ask you a question? During COVID, somebody discovered COVID shield and some other vaccine. I think Bharat Biologics had one, the UK had one.

What was that? That was really once they knew the sequence of the virus, they rapidly developed

the vaccine. So, once you know the cause of the disease, if you isolate the virus and you can understand its RNA sequence, it's like computer code. You can develop a virus without the harmful effects of the regular virus, and then develop a vaccine very rapidly.

Testing is what takes time. And during COVID, I think governments understood and accelerated the approvals and pushed the envelope in terms of not requiring very high number of trials over large numbers of people. They did some emergency policy making and developed a vaccine very fast.

In US politics, now you hear a lot of anti-vaxxers. Pfizer did this, they knew this, people getting heart attack, stroke, young people. My brother had a stroke, he's very healthy, 45-year-old.

Do you think there's any truth to that? I'm not scientifically qualified to say that, but there is a lot of noise about isolated strokes, heart attacks, coming from clots, which travel to the brain, based on the vaccines. But overall, if you see, without the vaccines, we wouldn't have gotten out of the COVID pandemic. You think? Yeah, 100% sure.

But once you have COVID, don't you develop some immunity? You do. It's like getting a vaccine? But the vaccine, the virus is mutating, and it gives you some protection, but you know, even people who got vaccinated, got some second dose and all. It's a whole different science which I don't fully understand.

But vaccines have played a big role in eliminating many diseases like polio, measles, many infectious diseases. Today, you know, I think the world is a much better place because of vaccines. You know, people today live till they are 70, 100 years ago they lived till they were 30.

Something has changed. Vaccines could be a part of this. Vaccines, nutrition, maternal health, a lot of things have increased longevity.

But any business opportunity, I keep going back to the 25-year-old, what is the opportunity here? Say, can I start a GLP-1 distribution business? Start a brand. I'll buy from you, I'll buy from you. I'll do marketing.

You know, what you could do is have everyday counseling for a guy who's on a GLP-1. GLP-1 is an everyday problem if you're on the GLP-1. Yeah? Yeah.

It causes bloating, it causes… So like an app that counsels people who are taking GLP-1? An app or even, you know, human to human counseling. You don't need to be a doctor to counsel? No. You can be a nutritionist, you can be a fitness… What if I find two fat people who became thin? I have a friend in mind.

Yeah. And they start a brand of GLP-1, buying the product from you guys? That is gone. Yeah? That is already mainstream now.

Yeah? Yeah. But I do great advertising. See, you give an example that this GLP has done this, but 20 brands are there of GLP, 20 good companies are giving goods.

What is the company's name of yours, your friends? Nothing is there. That is the point. On the contrary, I mean, this is a time when this, there is a wave of GLP, which is going on.

People want to cut down their weight, and they know it very well. When we see that, the muscle loss would be there. Right? Other factors are there.

So, there, that side can be promoted, and there some kind of a business can be developed. What we basically imagine, going forward, is that the sales of protein products, different kind of proteins, supplements, protein supplements, vitamins, minerals, other enzymes, coagulant, all sort of things, they will further increase. Because now people are cutting down the weight, knowing at least in the back of their mind that these are the problems going to happen.

So, I am supposed to go for some exercises, should I eat this? So, because I am doing this, so I should do that. So, some kind of a, I will say, pull effect would be there. Let me ask another question.

Is there no difference between Vigo, V, Monjaro? Monjaro is different. How is it different? It's a different molecule. Can't I take your GLP-1, add something, do something? No.

Don't please. So, you have really taken this FMCG too seriously. I think you have given him more confidence than he should have.

That's not a good idea. But I can do a counseling company along with all the other supplementation required into one. Since you're saying this, I'm just looking at what you do.

You've got a fantastic gym. You've got a chef who's measuring your calories and doing it. You're eating clean and all this.

This is something we don't get all of us get an opportunity to practice. You get. Others don't.

I maybe choose not to do it or something. Whatever. But you know, there are nutrition companies, there are companies which will tell you about fitness.

Why counseling would be very, very important because everybody knows it that there would come a time when he will stop taking these injections. And if after that he will not take certain precautions, whether eating right products, proteins, supplements, vitamins, this, this, this, and some kind of exercises, he will gain back faster. And this time, whatever he will gain will not be muscle, will be fat only.

Have you seen certain celebrities, look at their faces, what has happened? That's the problem. So point is what, if you take these things along with certain right kind of counseling and

precautions and certain supplements, this is a business. And add, add some kind of flavor in those.

Don't sell run-of-the-mill kind of brands in terms of anything and everything. Just go, I mean, somebody should go for a deep research and see what are the flaws in the present product, whether supplements, whether vitamins, and this, I mean, the only, the point is not only the effects, point basically is also, packaging plays a role, look and feel plays a role, taste plays a role. I mean, sometimes, when we have made a research, we found that lot of ladies during pregnancy time cannot take a calcium tablet because it is too big in size.

So can you cut down the size of the product? Can you add some kind of flavor in the coating because they have a nauseatic feeling always. So these are small things, but they are very important things. But you know, to sell these products, you need a salesforce, you need all this promotion.

It can't be done through advertising. Like, say, for example, I built a protein brand, it has worked. Can't I build a GLP-1 brand like that? No, you can't.

It's a prescription product. You can't do an ad. You can't do an ad, you can't talk to a patient.

We cannot talk to a patient. No, medicine can be promoted through advertisement. Even online you can't? No.

What if I see an American ad in India on YouTube? You can see that. That will not run here. On TV? On broadcast? No.

On YouTube? No. How is that possible? How can you sell an American product here in India? No, he's saying if Lilly has an ad on Munjaro, you can see it on YouTube here. Isn't Lilly sponsoring something in the IPL? There's some ad coming.

They're not saying, it's like how whiskey companies market soda, music. So I'll start a GLP1 company. I'll call it NKGLP.

I'll hire like a fat person who became thin to be the hero of it. And then after IPL match I'll say NKGLP ad. We make protein powder.

Is that a possibility? I'm just finding every edge case for a 25 year old. I say big no. No? The case against what you said is, it's illegal.

Right. In whiskey, it is probably grey. Why is government against it? Is there any part of the value chain that is finding its way to America even now after the patent is not there? No, no.

America, the patents are still on. No, but in India when you make it, do you have to pay them anything? No. We do it after patent expiry.

Then government doesn't like it because? Government doesn't like it because it's a pharmaceutical product. It has serious side effects. Correct.

And you can't promote it to people who are gullible. It's a muscle loss, problems in the bones. So many issues are there.

People are not basically looking for those side effects. I'll just tell you why I find this difficult to find an opportunity for a 25-year-old is, this is a deep science business. Right.

You have to have technical skills, all of that. Marketing is important, but the product is much more important. Right.

And you need to have the right products and the marketing overlay. But the 25-year-old is not making the product, he's buying it from you. He's building a brand on top of it.

If there was any arbitrage in that, I would have done it myself. There are many cases where there are arbitrages and people do it. See, I mean, this product, please understand that this product has to be prescribed only.

Even if a patient is buying it, right, it has to get a prescription of a doctor. It came in the US. They have something called compounding doctors, compounding pharmacies.

They actually buy the JLP powder from China and they mix it into an injectable, load it and ship it to customers. All for products which are under patent but are under supplied. So both Lilly and Nova had supply issues initially as the demand rose much higher than what they projected.

And these guys made billions of dollars. Young guys. All they had to do was contract manufacturing of some product, buy the ingredients, putting it together.

As a pharmaceutical company we don't want to do it because we are scared of them suing us. These guys made huge money. I know a guy who sold more than a billion dollars worth of product at 80-90% gross margin.

But you can't do that in India? No, you can't do it in India. There's a rule in the US that if there is a shortage, the pharmacist can compound it. Another question.

If you guys are operating at 20-25% EBITDA, which is quite high, why doesn't the, like you were talking about my chain, why doesn't the IN, the guy putting in the money to research, take over this profit center himself? So in many countries they do. So I'll tell you the IN and YANG in what context I told you that. The innovator is making, is spending billions of dollars, developing a product, launching it globally, and making a lot of money through a monopoly rights.

Patent rights give him monopoly rights for extended periods of time. Usually it's less than 10 years after accounting for all the time it takes to develop. Seven years, eight years.

And the profits are massive. They're selling at huge prices. And that is the innovator's business.

So he keeps on innovating and then doing it. As a generic company our approach is to get the patent life as short as possible. So we'll find a loophole to get an early, challenge the patents and say they're not valid patents.

So we fight that. So we try to shorten their useful life of their monopoly thing. They fight to extend it by suing us.

There's a constant legal battle. This is the IN and YANG. The people doing the generic cannot sustain without an innovator there because if there's no innovator there's no generic.

And if we don't bring their patents down, they'll have monopolies forever and they won't innovate fast enough. So in that sense the generic industry and the innovator industry are the IN and YANG of innovation. Why do Indian pharmacies, pharmaceutical companies, spend only three to five percent of budget on R&D but the Western ones spend 20? I think the nature of R&D that we do is different.

We do process development. We develop processes for products which are known and that doesn't require a huge amount of money. But some of us do research, we do spend, our numbers are double digit in R&D.

The bigger companies are spending, Sun spends, Zyder spends, all of these are double digit. Indian companies alone don't have to do it because they're focused only on India, they're doing branded generic business. For our India business we only develop products at a much lower cost, 2-3%, 5%.

If you're basically spending money on R&D, you're supposed to have global presence. Then only it will make some sense. Only from India, not possible.

Is that the plan in the future, like as a brand selling globally? See, all of us, we want to increase our margins. When you're sitting on the generic side, the innovation side looks so sexy. The margins are so good, the monopoly rights, swimming in money.

A company like Pfizer is what, 60-70 billion dollars in revenue. Merck is even bigger. That's bigger than the innovator.

But, innovation and generics, they're not a continuum. People like to say we're moving up the value chain, we're going to innovation. It's a very different business.

Just let me complete this train of thought, then you can come in. So, generics is something that you know, you develop processes, you file, you get, it's all very predictable. Discovery research and finding a novel therapeutic is a long, arduous process, very high failure.

So, you're moving in from a business which has a large number of small, tiny bets, to a business which has a very small number of very large bets. The risk profile is very different. So, to move there from here is a big challenge and that's the challenge that's something that we're all facing.

It requires much bigger capital and we can't leave this business because this is bread and butter and we need to keep funding that and also create a new business that is very demanding in terms of science and investment. This might be a stupid question, but when say whoever created GLP-1 created it, if you got the product, you can't reverse engineer it? That's what we do. That's what we did.

That's why we launched. That's the generic. That takes a long time? No, it takes two, three years.

We can do it. Maybe GLP-1 was a little more complicated product, so maybe four years. Otherwise, the day it got off patented, after one week it was launched.

When you say off patent, does it mean they disclose what the formulation is? No, no, no. We don't need that. Patents have all the details.

Expiry date in India was 26. So in the patent only, the formulation will be there? Yeah, but we don't even need that. We can reverse engineer from the product itself.

In this case, the Chinese are much better at biological processes, and the GLP-1 they make, the semaglutide they make, is from a biologic route. So they're able to scale it much better than Indians. Indian companies are largely in synthesis, which is an inefficient way of making semaglutide.

What else? Meanwhile, can you guys think of anything else for the 25-year-old? There are 128 unicorns in India today. There is one in pharma drug innovation, which is called Molbio. Molbio? Yeah.

Testing kit. The diagnostic company. Diagnostic company, yeah.

That's not even, you know, it's not in the same. Yeah, but you know, you do, it is innovative, what they did. The diagnostics they developed were much, much better and easier to, you know, it's tailored for the Indian market.

If you just look at this, you're talking about the R&D side in India, why companies are spending less? Because we are now coming to the cusp of that place where companies have become a bit serious. Why so? Because slowly and gradually, what basically is happening, government side, regulatory is becoming a bit stringent. Right.

More regulatory will become stringent. So, naturally, one day would come when every company, factory would be manufacturing a quality product. Right.

Then, the kind of advantage a branded company has, like I said, top 50 companies have more than 75% market share, we lose that gleam. Right. Because why some patient goes to a branded company, a known company, because there is a trust, that if I buy, I'm sure this is a right kind of product.

On the other side, because we keep hearing a lot of cases are there, that problem happened, this problem happened, and sometimes problem is not because of the drug which is there in the syrup or tablet. It is in the excipient which was used. Some kind of reaction happened, some problem came.

So, those kind of expertise are not there with those factories. Right. So, that fear is now there, at least if you ask me, in our head, that maybe what would happen out of 5, 10 years time.

So, why not to now invest and come to some kind of a level where we can rely on our own R&D. So, this is a big, see company like us, we spend 2.75% on R&D. But what we have decided now, every year we are supposed to increase it by 5.5%. Right.

And make it to the highest level possible. And then how smartly you use it. Right.

What kind of talent you bring. If you bring a right talent, they bring more talent. What is the CDMO industry? Is there an opportunity there? Yeah.

CDMO is doing well. The multiples are like hospital multiples now. Right.

What does it do? They do, they work for large innovative companies to either do FTE based research. Full time equivalence based research. Developing molecules, discovering libraries of compounds and all that.

Medicine, chemistry, or even biologics. That is one part. That is the CD part.

Manufacturing is actually producing the product. Right. Since these companies have very good margins and for them security of supply is much more important than cost at that point.

If you have a strong manufacturing infrastructure, quality systems, they will outsource the manufacturing to India. On top of this, the uncertainty with China, not just the tariff but also there is, what was that act called? The biosecure act. For their country's supply chain security, they passed a biosecure act which encouraged companies to move out of China.

There is a whole thing called China plus one strategy. That is an opportunity for Indian companies. All the CDMOs are growing very well.

Your own city has a nice one called Anthem Biotech. These are small companies. Anthem is big now.

They were small ones. They started with some small amount of capital. Today, I don't know the market cap is maybe 20,000 crores.

This is a good success. But someone small can try it. These guys were scientists working in Kiran's labs.

They came out and started somewhere and built a sizable business. Not 25 years old. You need to have scientific experience.

You can start when you are 25 and work your way through for the next 10 years. Can you think of anything? If you were a college teacher and you were teaching a class and you covered all that we covered today, what do you think do? I am diabetic. Ask Pragati if there is sugar in this.

But this protein powder with creatine and these nuts are really good. They are called butter pecan nuts. Okay, I'll try it.

And the base is tofu. Yeah, so what did we miss as a customer? In every part of our business, there are startups. There have been startups.

Like? Somebody who's starting at the age of 25, startup and some kind of a belief system in next five years time will make a mark. These are time taking things, a lot of patience. Once you basically talk about R&D, you talk about patience.

The first thing is that. Mankind is a good example of success. Yeah.

They took the traditional business model and disrupted it. They were seen as a disruptor. We were always telling our sales reps, see how much Mankind guy is doing, how come we are not growing at that pace.

Oh, they said, what they do, we can't do. I said, why not? They're this, they're that. Our compensation systems are different.

All kinds of excuses. But you came and turned it around. So there is something there to disrupt in all of our businesses.

Is there a patient feedback company somebody can start? Say, for example, I'm taking your Manforce whatever product or your GLP-1. What if somebody comes in and builds a company where there's an application where the patient every day reports back on video to an AI how he's feeling and I can sell the data back to you? Never heard about this. Worth thinking, but why a patient will give a feedback? My question is that.

What is the attraction to him? What would you pay to him? Maybe like, I don't know, 50 rupees a day. So question is that. I mean, will it be very expensive or not? And how will you monetize? The point is, I mean, somebody is taking a medicine, the effect comes out a few days.

It takes time. Fair. I would say there's still opportunity here.

I think we haven't figured out because we have our lens of large company, large size revenue streams. You talked a lot about AI everywhere. I tell you honestly, one thing which always haunts me.

Let me finish the AI part. So the AI part is a big deal for us and we are not equipped to do it, whether it's in our salesforce management, targeting doctors, all of that can be driven much more efficiently through agents and things like that. Our processes, R&D processes can improve.

Now we are able to do in silico experiments, in silico organoids. These are all areas where there can be services to pharma, which we don't have the capabilities to do. And we can't attract the talent because a smart AI guy doesn't want to work for a pharmaceutical company.

So you would be willing to outsource that? It's actually more than outsource. It's really a core process for us if we can do that. My idea is very simple.

Approximately 1.5 to 2% of medicines get expired. Where do these expired medicines go? In some place to be properly burned, destroyed, right? And this is totally unorganized. And there lies the scope.

Yeah? Yeah. So you'll pay money for somebody to take it from you and destroy it? Making sure that it does not come back in the market. That's a big problem, right? Counterfeits coming back.

It's an example. Every company basically has a pledge to take the expiry back and give the money back to the consumer. But what? I take the expiry, I burn it, I give it to some agency, it does not burn, it sells in the market, it again comes back.

Can some company give this kind of a guarantee properly? I mean, using technology, video, AI, whatever, that I am burning it. I give you a guarantee, I give you a certificate. All these things are there.

Because apart from this, government agencies are involved. Because when you burn some chemical, you pollute the area. So in a very remote place, certain red zones, you can open that.

But I tell you honestly, this is a good one. See, people always look at a glamorous side. They do this, they do what is going on.

Why don't you look at some basic, unglamorous thing? Which is basically need of the hour. I mean, sometimes you find that unglamorous things are more profitable. Everyone runs away glamorously.

I mean, looking at your idea about expiry medicines, in the US, I saw an NGO which does this. If the shelf life is less than some months, they take your medicines free of cost and distribute to

patients who need them. I mean, it's not a business, but it's philanthropy.

We need to have controls, but in the US, this is… What controls? I'm giving it to NGO. NGO is selling back, getting the money and then using the money somewhere else. Yeah, but I'm just saying this happened in the US.

I'm telling you, the idea is not bad. The idea is very good. I mean, six months before a product gets expired, it goes out of circulation.

What to do? And again, I mean, for example, let me talk about the expiry product. Very basic, very basic. Just a common sense.

At a 100 milligram, when it reaches 95, it is expired. Right? What if a poor patient takes 95 plus something, a bit more? Efficacy is 90. Who has no access to medicine.

It's a very wild thing I'm seeing. You know, this can go sideways if you do that. I'm not saying.

Can a product become poison? Yes, absolutely. It can? See, a milligram can cure you. That means a milligram can also hurt you.

My question is, you can tell me. Safety is important. For example, shelf life of a drug is two years.

Right? Example, if I take something after two years and two months. I wouldn't do that. I'll tell you why.

I'll give you my logic. The shelf life was arrived at the rigorous statistical analysis of all the data that the drug is good only for this time. You don't want to violate that.

And then the whole thesis of how you do business goes off. Six months before that, expiry. That's what I'm saying.

When they're short-dating, people are donating the medicines. Somebody can guarantee that? You can't test the drug to see if it's okay. Like a two and a half year old.

See, the government also needs some rules to enforce, right? They have given you two years lifetime based on the data you submitted. You can always extend it if you produce more data. Right.

And then say that, please extend it. And then you can do it. But without doing that, if you can't, you can't do anything.

I think that's not a right thing to do. I talk from business point of view. You talk from science point of view.

Not science. This is not science. This is rules, no? But it's nice.

You both have a good contradiction going. I talk from the idiot point of view. Anything else that you can think of for somebody young to start or think about? You see, I tell you honestly, when people start the business, the problem is what? They copy a set formula.

That this is the way these companies have become successful. And that is totally wrong. Never ever follow a set formula.

Example I give you. Leave it here, it's fine. I'll leave it there.

I'll keep it. The set formula was launch all India. You said no.

Only Western UP. Proof of concept in Western UP. Then expense loading gradually.

So idea of following somebody's set formula is always wrong. I say launch in a city. Very less risk.

That's a good lesson. Very good. See, first of all, you become a lion.

Become a lion in your own house first. That's a good strategy actually. We believe always focused approach.

100%. Find a mini market, succeed and then plan. Always.

And go in a market where nobody is going. Because there is always some place which can be competition free zone. Something will happen.

Somewhere. So just look at this e-commerce. When e-commerce came, so first it was Amazon and Flipkart.

Everybody came. Flopped. Closed down.

Then somebody said no, no. I'm only focusing narrowing down myself to cosmetics or clothing. Right? Or something else.

Then somebody said no. This is all bad. Quick commerce.

Did they follow? They took the inspiration but created a different path. So anytime, looking what is visible should not be seen. Should not be followed.

Ever. That we always follow. You know, if I had a bunch of genius AI guys, I would create a lab in my office for them and say this is the company, this is my cost structure, these are my cycle times, these are my problems.

It's not a pharma business but it's an AI business. If you had to ask your kids to join your business or to build something similar, would you? I would ask them to learn the business. And the business is of a much larger size than they will be able to run until they're in the business for a

decade.

And I don't think they have that kind of... One of my sons is in diagnostic business. The second has been put on the R&D side. I know my priorities.

If I say that next focus area of ours would be R&D, so I'm putting my bets over there. And diagnostics. Because going forward, this is going to play a big role.

Diagnostics? Home diagnostics. I mean, we sell Praga News. It's what? It's a kit just to check your pregnancy.

How much we sell? Yeah. 300 crore rupees worth? I think the game changer will be when you can take blood on your own. Yeah.

Somebody was trying that. You can still take a drop. People do it for measuring their blood sugar.

Microfluidics are coming in, which can give a lot more data with just that one look. Like that Theranos. Theranos was a fraud but that's the idea.

But the principle. Yeah. That's possible.

It's possible. Like in one drop of blood I can find out my liver enzyme, my cholesterol, all of that. There are people trying to do cancer with that.

One drop. Some screaming. I think that's it.

We recorded for four and a half hours, which is a long time. I'm not sure you've got something your audience would be interested in. No, no.

It's interesting. See, the thing is, it's not even that here is the option you should go do it. It's also figuring out what can't be done.

It's useful. So if you don't think of it as a fast build business, this is a great business to be in. But you should have a decade to build it.

Yeah. A good example is Anthem. 32,000 crores.

I was an employee. Any last words? You guys want to say something to the audience? There's nothing shortcut. I mean, you have to be very, very patient if you want to come in this line.

And the best way to start your business is first to take some experience in some area where you want to basically focus later on. It's always good, I mean, without risking your money. Right.

You're getting the experience. I'd say that the pharmaceutical business is great business for India. If you take out the IT sector, this is a business which takes products to all over the world.

It's sold as branded products in many parts of the world. And I think huge impact. So I'm very proud of what we have done, and I think there's more that we can do.

Pharmacy of the world doesn't happen very fast. Of course, it's a little bit generic pharmacy of the world. But, you know, it's a good foundation to build on.

I think the next frontier is finding new drugs. I feel, I mean, next 10 years would be very interesting. Extremely interesting for Indian pharma industry.

Because once you know that if you basically are cognizant to the future challenges, you take proactive actions. Super. Thank you so much for doing this.

I hope you didn't get too bored. No, no. Thank you very much.

Thank you. Thanks all. Thank you very much.

Middle of May. This was a different kind of a dinner podcast. Yeah.

It's very college-like. Chat and talk. When we were researching you, we learned that you like taking photographs of birds.

Yes. We would have never expected it. What is the story? I, you know, from my childhood, I was always interested in nature.

It's not photography is my interest, nature is my interest. So, one of my friends who lives down the road was a businessman and a very good ornithologist. So, he introduced me to birds and that's how I started birding.

Then it became a passion. And I published some books and I also go on trips for birding. What's your idea of a fun weekend? Saturday, Sunday, what would you do? I don't know if it's about fun, but if I had an ideal weekend, I would go to the farm or something.

And the weather is not like this, but it's just nice. And I shoot some pictures. I visit my parents, my mother.

My father has passed away. My mother, spend some time with her and then maybe in the evening, meet some friends. It's very interesting.

And for you? For me, the idea of fun. Fun weekend. Friends, drinking.

In Delhi. Anyplace.